



State of Utah

SPENCER J. COX
Governor

DEIDRE M. HENDERSON
Lieutenant Governor

Insurance Department

JONATHAN T. PIKE
Insurance Commissioner

2025 Utah Health Insurance Market Report

Table of Contents

Table of Contents	2
List of Tables	4
List of Figures	6
Executive Summary	8
Introduction	9
What is Health Insurance?	9
Estimate of Comprehensive Health Insurance Coverage in Utah	10
Utah’s Commercial Health Insurance Market	11
Commercial Health Insurance Market Overview	11
Commercial Health Insurance Market by Policy Type	13
Consumer Complaints Against Commercial Health Insurers	14
Independent Reviews	18
Utah’s Comprehensive Health Insurance Market	19
Comprehensive Market by Domicile	20
Comprehensive Health Insurance Market by Group Size	20
Comprehensive Market by Plan Types	21
Comprehensive Health Insurance Market by Regulatory Type	22
Comprehensive Health Insurance Market Trends	25
Health Care Preauthorization Reporting	35
Utah’s Stop-Loss Insurance Market	36
Stop-Loss Insurance Market Trends	36
Stop-Loss Insurance Market by Domicile	36
Stop-Loss Insurance Market by Group Size	37
Stop-Loss Insurance Market by Attachment Points	37
Utah’s Long-Term Care Insurance Market	39
Long-Term Care Market by Domicile	39
Long-Term Care Market by Group Size	40
Long-Term Care Market by Age	40

Utah’s Medicare Product Market	41
Medicare Products by Age	42
Medicare Products by Plan Type	43
Pharmacy Benefit Manager Rebates	44
Conclusion	45
References	46
Recommendations	50
List of Health Insurance Mandates in Utah	52
Coverage Mandates	52
Benefit Mandates	53
Provider Mandates	53
Statutory Requirements and Methods Overview	54
Statutory Requirements	54
Methods Overview	55
Glossary	57

List of Tables

Table 1. Estimate of Health Insurance Coverage for 2024	11
Table 2. Total Commercial Health Insurance Market by Insurer Type for 2024	12
Table 3. Total Commercial Health Insurance Market by Policy Type for 2024	13
Table 4. Number of Consumer Telephone Contacts Handled by OCHA Staff: 2015 - 2024	14
Table 5. Complaints Filed with OCHA by Type: 2015 - 2024	15
Table 6. Complaints Filed with OCHA by Reason: 2015 - 2024	16
Table 7. Complaint Ratios for the Commercial Health Insurance Market: 2015 - 2024	16
Table 8. Commercial Health Insurance Companies with Consumer Complaints in 2024	17
Table 9. Requests for Independent Reviews by Eligibility: 2015 - 2024	18
Table 10. Requests for Independent Reviews by Reason: 2015 - 2024	18
Table 11. IRO Decisions by Outcome: 2015 - 2024	19
Table 12. Total Comprehensive Market by Domicile for 2024	20
Table 13. Total Comprehensive Market by Group Size for 2024	21
Table 14. Total Comprehensive Market by Plan Type for 2024	22
Table 15. Total Comprehensive Market by ACA Market Segment for 2024	23
Table 16. Metal Tier Plans on Federally Facilitated Marketplace for 2024	24
Table 17. HSA-Qualified High Deductible Health Plans for 2024	24
Table 18. Changes in the Number of Comprehensive Health Insurers: 2015 - 2024	25
Table 19. Changes in Comprehensive Membership by Group Size: 2015 - 2024	27
Table 20. Changes in Comprehensive Membership by Plan Type: 2015 - 2024	28
Table 21. Changes in Government Sponsored Health Benefit Plans: 2015 - 2024	
Table 22. Comprehensive Premium Compared to National Economic Trends: 2015 - 2024	29
Table 23. Comprehensive Losses Compared to National Health Care Spending: 2015 - 2024	30

Table 24. Changes in Comprehensive Premium and Per Capita Income: 2015 - 2024	31
Table 25. Comparison of Utah Premium to National Premium: 2015 – 2024	32
Table 26. Health Care Preauthorizations processed after one week in 2024	34
Table 27. State Benefit Mandate Defrayal Payments for Autism Behavioral Health Treatment: 2020 - 2024	35
Table 28. Total Stop-Loss Market: 2015 - 2024	36
Table 29. Total Stop-Loss Market by Domicile for 2024	36
Table 30. Total Stop-Loss Market by Group Size for 2024	37
Table 31. Stop-Loss Membership by Specific Attachment Points for 2024	38
Table 32. Stop-Loss Membership by Aggregate Attachment Points for 2024	39
Table 33. Total Long-Term Care Market by Domicile for 2024	39
Table 34. Total Long-Term Care Market by Group Size for 2024	40
Table 35. Long-Term Care Membership by Age for 2024	40
Table 36. Total Medicare Supplement Market by Domicile for 2024	41
Table 37. Total Medicare Advantage Market by Domicile for 2024	41
Table 38. Total Medicare Part D Market by Domicile for 2024	42
Table 39. Medicare Supplement Membership by Age for 2024	42
Table 40. Medicare Advantage Membership by Age for 2024	42
Table 41. Medicare Part D Membership by Age for 2024	42
Table 42. Medicare Supplement Membership by Plan Type for 2024	43
Table 43. Medicare Advantage Membership by Plan Type for 2024	43
Table 44. Pharmacy Benefit Manager Rebates and Administrative Fees for 2024	44
Table 45. List of Comprehensive Health Insurers in 2024	51

List of Figures

Figure 1. Estimate of Health Insurance Coverage for 2024	10
Figure 2. Comprehensive Premium PMPM by Group Size: 2015 - 2024	32
Figure 3. Income After Expenses For Comprehensive Health Insurers: 2015 - 2024	34

The *2025 Health Insurance Market Report* was published by the Health and Life Division of the Utah Insurance Department. Publication date: February 1, 2026. For questions about this report, please contact the Health & Life Division at (801) 957-9280 or healthresearch@utah.gov.

Executive Summary

- In 2024, approximately 41 percent of Utah's commercial health insurance market was from comprehensive health insurance.
- The comprehensive health insurance industry served 24.2 percent of Utah residents in 2024, up from 23.1 percent in 2023.
- In 2024, the total number of consumer complaints to the Insurance Department (Department) increased by 46 percent over 2023, from 230 to 336.
- From 2023 to 2024, the number of requests for independent reviews increased by 69 percent, from 233 to 393.
- Six comprehensive health insurers participated in the Federally Facilitated Marketplace (FFM) in 2024.
- In 2024, comprehensive health insurance enrollment increased by about 7.2 percent. This increase appears to be due to changes in the large group market.
- Large group membership increased 30.4 percent from 2023 to 2024, while individual and small group markets decreased.
- Comprehensive health insurance premium per member per month (PMPM) increased from \$429 in 2023 to \$456 in 2024. This is an increase of 6.3 percent.
- The national average of comprehensive health insurance premium PMPM slightly increased from \$524 in 2023 to an estimated \$525 in 2024.
- In 2024, Utah's comprehensive health insurance premiums were 15.1 percent lower than the national average.
- The top 7 insurers in the comprehensive health insurance industry have experienced an average net income after expenses of 2.0 percent over the last ten years, with the top comprehensive health insurers reporting an average gain of 3.5 percent in 2024.
- On average, the percentage of health care preauthorizations processed more than one week after the day on which the insurer received the preauthorization request was 9.7 percent.
- The Department has developed a list of recommendations for legislative consideration that could improve Utah's health insurance market. These are reported in the Appendix (see page 50).

Introduction

Utahns receive their health insurance coverage in three ways: plans sponsored by the government, employers, and commercial health insurers. This report's focus is on commercial health insurance, which is governed by state and federal law and is the only source of health insurance regulated by the Utah Insurance Department (referred to as the Department in this report). Commercial insurance health plans can be issued as fee-for-service plans, nonprofit health service plans, health maintenance organizations, and limited health plans. The health insurance benefits range from comprehensive major medical health insurance to limited benefits such as dental, vision, or specified disease.

Health insurance is the primary way healthcare costs are covered. It protects against the risk of financial loss from accidents, illness, chronic health problems, and routine medical interventions. Because health insurance is so important to Utah's citizens, it is in the State's interest to monitor and maintain a stable health insurance industry.

The Department strives to ensure that Utah has an adequate and healthy insurance market. This report provides an annual evaluation of Utah's commercial health insurance market as required by Utah Code § 31A-2-201.2.

What is Health Insurance?

Health insurance transfers the risk of paying for personal health care from an individual to a company that pools the risk. The individual shares in the cost of their health care through deductibles, coinsurance, and insurance coverage. Individuals can obtain their health coverage from several sources, including government sponsored health plans, employer sponsored self-funded health plans, and group or individual commercial insurance health plans. The coverage provided ranges from comprehensive major medical benefits to single disease or accident only benefits.

Government sponsored health plans are programs that may be funded entirely by government funds or by a combination of government funds and premiums paid by the individuals enrolled in the program. The risk of financial loss is taken on by the government. These programs may provide comprehensive major medical health benefits (such as Medicaid and Medicare), limited primary health benefits (such as county health clinics), or limited specialized health benefits.

Government sponsored health plans are regulated by Federal regulatory agencies like the Centers for Medicare and Medicaid Services (CMS). These programs do not include individual comprehensive health insurance benefits purchased through the Federally Facilitated Marketplace ([healthcare.gov](https://www.healthcare.gov)), which may be funded in part by federal tax credits.

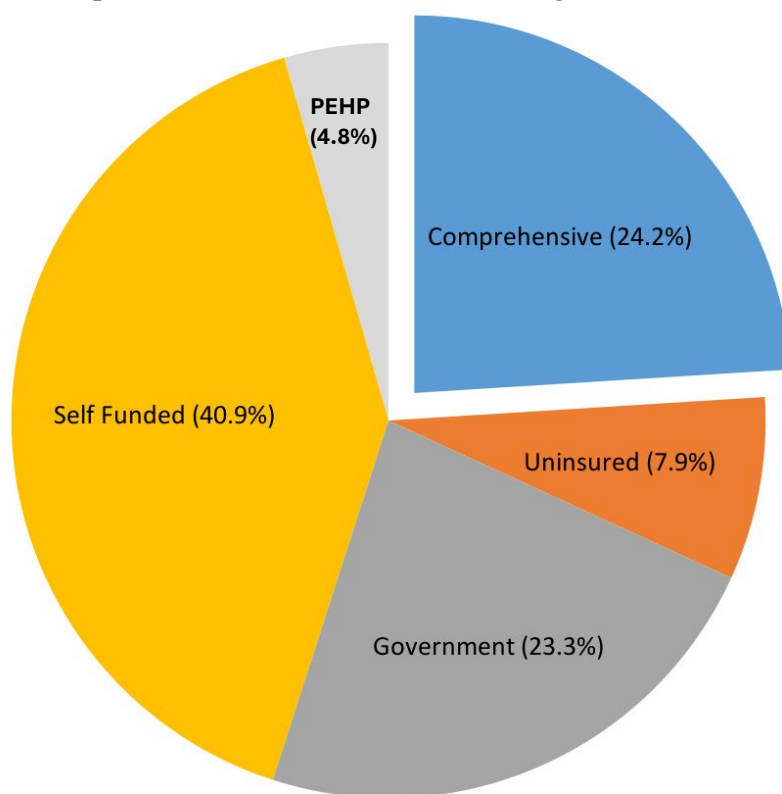
Employer sponsored self-funded health plans provide health coverage to the enrolled company's employees. These plans may be funded entirely by the employer or by a combination of employer funds and amounts withheld from covered employees' wages. The risk of financial loss is taken on by the employer. However, most employer sponsored self-funded plans purchase

commercial stop-loss insurance coverage for added protection. These plans usually provide comprehensive health coverage and may provide benefits only to the employee, or to the employee and the employee's dependents.

Estimate of Comprehensive Health Insurance Coverage in Utah

Based on available information, the Department has estimated the extent of insurance coverage in the State by each source of health insurance (Figure 1). This estimate is only for comprehensive health insurance coverage (also known as major medical). See Table 1 for additional details.

Figure 1. Estimate of Comprehensive Health Insurance Coverage for 2024



Data Sources: CMS, Deseret Mutual Benefit Administrators, Public Employee Health Program, Utah Department of Health and Human Services, Utah Insurance Department, and the U.S. Census Bureau.

Note: The estimate of the 2024 employer sponsored self-funded membership is based on limited data from commercial insurers and employers. It is not a complete count of the self-funded membership in Utah and should be used with caution. Estimates may not total exactly due to rounding and differences in methodology.

Multiple data sources with different methods were used to create this estimate. Membership data for government sponsored health benefit plans was obtained from the Utah Department of Health and Human Services (DHHS) and CMS. Membership data for comprehensive health insurance were obtained from the Utah Accident & Health Survey, an annual survey conducted by the Department. The estimate for the uninsured was obtained from the U.S. Census Bureau.

Currently, there is no single source of data for employer sponsored self-funded plans in Utah.

Because of this, the Department created an estimate based on a combination of data obtained from government sponsored plans, large self-funded employers, and commercial health insurers who administer self-funded health benefit plans. The result, while not exact, does provide an estimate of the employer sponsored self-funded population.

Given these limitations, the Department estimates that in 2024, 23.3 percent of Utahns were covered by government plans, 45.4 percent were covered by employer sponsored self-funded plans, and an estimated 7.9 percent were uninsured (see Table 1). The percent of Utahns covered by comprehensive health insurance increased from 23.1 percent in 2023 to 24.2 percent in 2024.

Table 1. Estimate of Comprehensive Health Insurance Coverage for 2024

Coverage Type	Population Estimate	Percent of Population
Government Sponsored Plans	815,575	23.3%
Medicare	465,601	13.3%
Medicaid	337,462	9.6%
Children’s Health Insurance Program (CHIP)	12,512	0.4%
Employer Sponsored Self-Funded Plans	1,590,921	45.4%
Plans Administered by Commercial Insurers	878,432	25.0%
Public Employee Health Program (PEHP)	158,308	4.5%
Federal Employee Health Benefit Plan (FEHBP)	122,661	3.5%
Other Self-Funded Plans (estimated)	431,520	12.3%
Commercial Health Insurance Plans	847,655	24.2%
Group	441,837	12.6%
Individual	405,818	11.6%
Uninsured Estimate	277,000	7.9%
Total Utah Population	3,506,838	100.0%

***Data Sources:** CMS, Deseret Mutual Benefit Administrators, Public Employee Health Program, Utah DHHS, Utah Insurance Department, and the U.S. Census Bureau.*

***Note:** The estimate of the 2024 employer sponsored self-funded membership is based on limited data from commercial insurers and employers. It is not a complete count of the self-funded membership in Utah and should be used with caution. Estimates may not total exactly due to rounding and differences in methodology.*

Utah’s Commercial Health Insurance Market

Commercial insurers manage risk. They accept the risk of loss to individuals or organizations in exchange for a premium. In doing so, the risk of loss is shared (or pooled) so that one individual does not bear all the risk of loss.

Insurance companies that operate in Utah report financial data to the Department and the National Association of Insurance Commissioners (NAIC). This data includes health insurance premium data about individual and group policyholders, as well as government sponsored programs such as Medicare and Medicaid. The premium reported does not include fees paid to insurers for the administration of self-funded health benefit plans.

Commercial Health Insurance Market Overview

Among commercial health insurers, there are different products. They can include comprehensive health insurance, specialized coverage such as long-term care, dental, vision, disability, accident, specified disease, or as a supplement to other kinds of health plans. At the end of 2024, there were 1,466 commercial fraternal, life, health, and property and casualty insurers licensed in Utah with the Department. Of these, 319 reported health insurance business on their 2024 annual financial statements. These insurers represent the commercial health insurance policies sold in Utah. Each commercial insurer reported direct premiums and incurred losses in Utah, as well as total revenue and net income.

Table 2 summarizes some characteristics of Utah’s commercial health insurance market, as well as some national figures. As a group, Utah’s commercial health insurers had a loss ratio of 85.6. The loss ratio compares incurred losses to the premiums earned. A ratio of less than 100 indicates that an insurer received a higher premium income than it paid out in claims. A ratio of more than 100 indicates that an insurer paid more in claims than it received in premium income. Commercial health insurers generally aim to maintain a loss ratio of less than 85 (85 cents in losses for every dollar of premium). If the loss ratio increases much beyond 85, an insurer may incur more expenses than income and suffer a financial loss.

The state of domicile is important to consider in Utah’s commercial health insurance market. Domestic insurers have a home office in Utah. Foreign insurers have a home office in another state. About 73 percent of Utah’s commercial health insurance market is domestic. These 24 domestic insurers are much more representative of the Utah market, as about 70 percent of their total revenue comes from Utah business. Because of this, their loss ratios and net income are more accurate measures of the Utah market. As a group, domestic insurers had a loss ratio of 88 and a net income of 2 percent.

There were 295 foreign insurers in Utah’s commercial health insurance market in 2024, most of which are life insurers. These foreign insurers account for about 27 percent of Utah’s market. Foreign insurers had a loss ratio of 70.4 for Utah business. Net income was 5 percent of total revenue; however, less than 1 percent came from Utah businesses, indicating they are not representative of Utah (see Table 2). Overall, foreign insurers have a relatively small presence in Utah’s commercial health insurance market.

Table 2. Total Commercial Health Insurance Market by Insurer Type for 2024

Insurer Type	Utah Operations				National Operations	
	Company Count	Direct Earned Premium	Market Share	Loss Ratio	Total Revenue	Net Income (% Rev)
Domestic Insurers						
Health	12	\$8,036,858,713	71.7%	88.0	\$10,060,674,314	2.6%
Life	10	\$139,042,955	1.2%	90.9	\$1,623,774,815	-1.9%
Limited Health Plan	2	\$1,665,980	0.0%	48.5	\$1,665,980	21.1%
Total Domestic	24	\$8,177,567,648	73.0%	88.0	\$11,686,115,109	2.0%
Foreign Insurers						
Fraternal	13	\$760,866	0.0%	64.0	\$16,571,322,672	6.5%
Life	236	\$2,976,451,927	26.6%	70.5	\$1,074,012,671,190	3.1%
Property & Casualty	46	\$54,316,607	0.5%	65.1	\$698,894,980,238	7.7%

Total Foreign	295	\$3,031,529,400	27.0%	70.4	\$1,789,478,974,100	5.0%
Utah Insurers						
Fraternal	13	\$760,866	0.0%	64.0	\$16,571,322,672	6.5%
Health	12	\$8,036,858,713	71.7%	88.0	\$10,060,674,314	2.6%
Life	246	\$3,115,494,882	27.8%	71.5	\$1,074,012,671,190	3.1%
Limited Health Plan	2	\$1,665,980	0.0%	48.5	\$1,665,980	21.1%
Property & Casualty	46	\$54,316,607	0.5%	65.1	\$698,894,980,238	7.7%
Total Utah	319	\$11,209,097,048	100.0%	85.6	\$1,799,541,314,394	5.0%

Data Source: NAIC Financial Database

Note: The total direct earned premium and total revenue reported is based on the annual financial statement data submitted by commercial insurers to the National Association of Insurance Commissioners (NAIC). Estimates may not total exactly due to rounding.

Commercial Health Insurance Market by Policy Type

Financial statement data is used to measure the financial solvency of commercial insurers and does not provide details about specific types of insurance. To get detailed information about Utah's commercial health insurance market, insurers are required to submit the Utah Accident & Health Survey. Data was submitted by 319 commercial health insurers who reported accident & health premiums in Utah in 2024.

The top four policy types by market share were comprehensive health insurance (41.2 percent), Medicare Advantage products (30.2 percent), Medicaid/CHIP (7.6 percent), and Federal Employee Health Benefit Plan (FEHBP) (7.2 percent) (see Table 3).

Table 3. Total Commercial Health Insurance Market by Policy Type for 2024

Policy Type	Insurer Count ^a	Member Count ^b	Direct Earned Premium	Market Share	Loss Ratio
Comprehensive	27	847,655	\$4,614,989,212	41.2%	85.4
Hospital-Medical-Surgical	25	49,667	\$9,815,043	0.1%	10.8
Short-Term Limited Duration	7	8,828	\$12,853,579	0.1%	114.0
Medicare Supplement	98	79,785	\$217,675,600	1.9%	83.4
Medicare Advantage	15	248,662	\$3,385,616,241	30.2%	88.3
Medicare Drug Plan	6	62,438	\$61,910,336	0.6%	91.3
Dental Only	59	717,480	\$266,657,282	2.4%	85.4
Vision Only	38	774,484	\$51,895,506	0.5%	67.7
FEHBP	6	122,661	\$805,506,275	7.2%	93.4
Medicaid/CHIP	2	179,543	\$849,029,603	7.6%	90.2
Stop-Loss	39	648,749	\$391,125,926	3.5%	101.3
Disability Income	105	841,206	\$277,786,265	2.5%	62.0
Long-Term Care	60	28,244	\$42,739,699	0.4%	109.7
Credit A&H	12	104,893	\$9,988,099	0.1%	19.6
All Other A&H	179	-	\$211,508,382	1.9%	21.4
Total Accident & Health	319	-	\$11,209,097,048	100.0%	85.6

Data Source: Utah Accident & Health Survey

Note: The FEHBP, Medicare, and Medicaid business reported here may include some health benefit plans that are not fully insured as NAIC accounting rules allow certain types of administrative business to be

reported on the state page of the annual statement. These categories are included here to ensure that the accident & health business being reported in the Utah Accident & Health Survey is consistent with the accident & health business being reported on the Utah state page of the NAIC annual statement. Estimates may not total exactly due to rounding.

^a Insurer count column does not add up to the total because an insurer may have more than one policy type.

^b A total is not reported for the column "Member Count" and for "Other." People can be enrolled in more than one type of insurance so totals aren't reported to avoid double counting.

Consumer Complaints Against Commercial Health Insurers

Another function of the Department is to help people with questions or concerns about commercial health insurance coverage. The Office of Consumer Health Assistance (OCHA) within the Department provides services to health care consumers and policymakers. These services include, but are not limited to:

- Helping people understand their rights and responsibilities, statutory protections, and available solutions under their health plan
- Providing education about health insurance
- Investigating and resolving complaints for health insurance
- Assisting those who have difficulty accessing their health insurance benefits because of language, disability, age, or ethnicity
- Providing people with information and help initiating the grievance process for denied services
- Analyzing and monitoring federal and state regulations that apply to health care consumers
- Facilitating independent reviews

Consumers contact OCHA for a variety of reasons. These contacts range from simple questions about how to obtain health insurance coverage to complaints against a particular health insurer. OCHA engaged in approximately 7,000 telephone calls with consumers in 2024. In addition to telephone contacts, OCHA staff also respond to a wide variety of health and life cases based on the needs of the consumer. These types of cases include general inquiries, complaints about health and life insurance, independent reviews, investigations, complaints against employer sponsored self-funded plans, and policies issued in other states where the Department does not have jurisdiction.

Table 4. Number of Consumer Telephone Contacts Handled by OCHA Staff: 2015 - 2024

	2015	2016	2017	2018	2019	2020	2021	2022	2023	2024
Telephone (in/out)	4,369	6,892	5,685	8,349	9,691	7,274	7,357	6,385	7,905	7,013
Total Cases ^a	-	-	-	-	1,115	1,043	846	1,013	1,258	1,557

Data Source: Utah Insurance Department

Note: Due to a software limitation in the Utah Insurance Department's phone system from 2020 - 2024, this data may not include all phone calls received by an employee's direct line.

^aTotal cases are reported for 2019-2024. All data is current as of Dec. 31 of the report year.

When a consumer contact involves a possible violation by a commercial health insurer of state or federal health insurance laws or regulations, OCHA encourages consumers to file a written complaint. Once submitted, OCHA investigates and works to resolve the issue. OCHA tracks all

written complaints made against commercial health insurers. They are classified into two types: confirmed and unconfirmed.

Confirmed vs. Unconfirmed. Confirmed complaints are those where the Department determines that the complaint is warranted under the law. It is resolved by requiring the commercial health insurer to act to correct the problem. Unconfirmed complaints are those where the Department rules in favor of the commercial health insurer. This could result from the insurer being found acting within the law, or from the Department being unable to make a ruling. The latter may occur due to unresolved questions in the case or because the Department lacks legal authority to do so. In these situations, the Department educates consumers about their rights and how health insurance contracts work.

Table 5 shows the number of complaints filed with the Department each year. While the OCHA staff and the Utah health insurance industry work hard to resolve concerns before they become a formal written complaint, the number of confirmed and unconfirmed cases have both increased from 2020 to 2024.

Table 5. Complaints Filed with OCHA by Type: 2015 - 2024

Year	Confirmed		Unconfirmed		Total
	Count	Percent of Total	Count	Percent of Total	Count
2015	136	48.6%	144	51.4%	280
2016	140	40.7%	204	59.3%	344
2017	85	26.2%	239	73.8%	324
2018	46	17.4%	219	82.6%	265
2019	28	13.0%	187	87.0%	215
2020	12	8.2%	134	91.8%	146
2021	22	12.4%	155	87.6%	177
2022	23	10.8%	190	89.2%	213
2023	29	12.6%	201	87.4%	230
2024	44	13.1%	293	86.9%	337
Average	56.5	22.3%	196.6	77.7%	253.1

Data Source: Utah Insurance Department

Over the last 10 years, the number of complaints about balance billing has increased both in Utah and nationwide. This is when a health care provider bills the patient for the difference between the provider's charge and the amount paid by health insurance. The federal government passed the No Surprises Act (2021) to address this matter. This law provides protection from surprise medical balance billing. This federal law applies to individual and group health benefit plans and took effect on January 1, 2022.

In addition to tracking the number of written complaints and how they are resolved, the Department also tracks the reason for the complaint. As shown in Table 6, on average, over 70 percent of all consumer complaints are due to claim handling issues, while policyholder services and marketing & sales issues account for most of the remainder. In 2024, four complaints were also filed regarding underwriting issues.

Table 6. Complaints Filed with OCHA by Reason: 2015 - 2024

Year	Claim Handling		Policyholder Services		Marketing & Sales		Total
	Count	Percent of Total	Count	Percent of Total	Count	Percent of Total	Count ^a
2015	174	62.1%	89	31.8%	17	6.1%	280
2016	200	58.1%	130	37.8%	14	4.1%	344
2017	239	71.6%	90	26.9%	5	1.5%	334
2018	196	70.3%	75	26.9%	8	2.9%	279
2019	165	72.1%	50	21.8%	14	6.1%	229
2020	121	81.8%	27	18.2%	0	0.0%	148
2021	122	64.6%	53	28.0%	14	7.4%	189
2022	171	75.3%	19	8.4%	37	16.3%	227
2023	186	76.9%	41	16.9%	15	6.2%	242
2024	217	64.4%	71	21.1%	43	12.8%	335
Average	179.1	70.9%	64.5	25.6%	16.7	6.6%	252.4

Data Source: Utah Insurance Department

Note: Policyholder Services includes complaints regarding policyholder services and underwriting practices. Estimates may not total exactly due to rounding.

^aIn 2024, 4 cases were filed for issues with underwriting.

Another measure of complaint activity is the complaint ratio. This is a measure of the number of consumer complaints received relative to the amount of business a commercial health insurer did in the state. Table 7 reports the complaint ratios for the commercial health insurance market from 2015 to 2024. Each complaint ratio reports the number of complaints per \$1,000,000 in total direct earned premium. For example, a ratio of 1 means the insurer had 1 complaint for every \$1,000,000 in premium. As Table 7 shows, the average complaint ratio for the commercial health insurance market is 0.02 for all complaints, 0.01 for confirmed complaints, and .02 for unconfirmed complaints. Additionally, there were 11 complaints which were a question of fact. This category was added in 2024 when the criteria for each category was revised.

Table 7. Complaint Ratios for the Commercial Health Insurance Market: 2015 - 2024

Year	Confirmed		Unconfirmed		Total		Direct Earned Premium
	Count	Ratio	Count	Ratio	Count	Ratio	
2015	136	0.02	144	0.03	280	0.05	\$5,705,636,933
2016	140	0.02	204	0.03	344	0.06	\$6,215,575,220
2017	85	0.01	239	0.04	324	0.05	\$6,577,788,210
2018	46	0.01	219	0.03	265	0.04	\$7,134,644,985
2019	28	< 0.01	187	0.03	215	0.03	\$7,421,241,744
2020	12	< 0.01	134	0.02	146	0.02	\$8,307,738,287
2021	22	< 0.01	155	0.02	177	0.02	\$8,822,179,189
2022	23	< 0.01	190	0.02	213	0.02	\$9,662,122,005
2023	15	< 0.01	151	0.01	166	0.02	\$10,425,904,226
2024	138	0.01	187	0.02	336 ^a	0.03	\$11,209,097,048

Average	55.1	0.01	191.6	0.02	246.7	0.03	\$8,148,192,785
----------------	-------------	-------------	--------------	-------------	--------------	-------------	------------------------

Data Sources: NAIC Financial Database and the Utah Insurance Department

^a 11 cases in 2024 were determined to be a question of fact

Table 8 reports individual complaint ratios for commercial health insurance companies in 2024. The averages in Table 7 can provide perspective on these individual ratios. For example, a commercial health insurer with a total complaint ratio of greater than 0.03 has a higher-than-average number of complaints, while a ratio of less than 0.03 means a lower-than-average number of complaints. It is also important to remember that a complaint ratio is only one aspect of evaluating a commercial health insurer.

Table 8. Commercial Health Insurers with Consumer Complaints During 2024

Insurer Name	Direct Earned Premium	Market Share	Confirmed		Unconfirmed		Total	
			Count	Ratio	Count	Ratio	Count	Ratio
SelectHealth Inc	\$4,136,133,337	36.9%	32	0.01	37	0.01	69	0.02
Regence BlueCross BlueShield of Utah	\$1,361,669,323	12.1%	6	0.00	13	0.01	19	0.01
Molina Healthcare of Utah, Inc	\$466,602,417	4.2%	2	0.00	5	0.01	7	0.02
United Healthcare Insurance Company	\$423,675,341	3.8%	13	0.03	19	0.04	32	0.08
Aetna Life Insurance Company	\$369,275,410	3.3%	18	0.05	14	0.04	32	0.09
Humana Insurance Company	\$345,574,030	3.1%	1	0.00	2	0.01	3	0.01
Cigna Health and Life Insurance Company	\$308,954,410	2.8%	4	0.01	9	0.03	13	0.04
Principal Life Insurance Company	\$234,899,373	2.1%	2	0.01	1	0.00	3	0.01
University of Utah Health Insurance Plans	\$122,166,742	1.1%	2	0.02	3	0.02	5	0.04
Metropolitan Life Insurance Company	\$75,421,483	0.7%	3	0.04	3	0.04	6	0.08
Ameritas Life Insurance Corp.	\$41,722,864	0.4%	8	0.19	8	0.19	16	0.38
Educators Mutual Insurance Association	\$35,192,952	0.3%	3	0.09	2	0.06	5	0.14
National Foundation Life Insurance Company	\$31,105,370	0.3%	1	0.03	5	0.16	6	0.19
Guardian Life Insurance Company of America	\$27,530,602	0.2%	0	0.00	5	0.18	5	0.18
National Health Insurance Company	\$20,290,336	0.18%	1	0.05	2	0.10	3	0.15
Renaissance Life & Health Insurance Company of America	\$12,560,099	0.11%	4	0.32	1	0.08	5	0.40
Prudential Health Insurance Company of America	\$12,129,442	0.1%	1	0.08	2	0.16	3	0.25
17 Companies with 3 or more complaints^b	\$8,024,903,531	71.6%	101	0.01	131	0.03	232	0.03
Remaining companies^a	\$3,184,193,517	28.7%	37	0.00	56	0.00	93	0.03
Total Commercial Health Insurance Market	\$11,209,097,048	100.0%	138		187		336^c	

Data Sources: NAIC Financial Database and the Utah Insurance Department

Note: Estimates may not total exactly due to rounding.

^a Remaining companies include 60 companies with 1 or 2 complaints and 242 companies with no complaints.

^b Includes all companies with 3 or more complaints

^c 11 cases in 2024 were determined to be a question of fact

Independent Reviews

In addition to consumer complaints, the Department receives independent review requests from consumers about denied claims. These reviews are conducted by an Independent Review Organization (IRO). An independent review can be filed after the consumer has gone through the standard claim appeals process with their commercial health insurer.

When the Department receives a request for an independent review, it is assigned to an IRO. However, not all denied claims are eligible for an independent review. If a claim is not eligible for review, it did not meet the minimum criteria. This is often because the denied claim was not covered under the consumer's health insurance policy. Claims eligible for review were typically denied because the insurer determined they were experimental or not medically necessary. If a claim is eligible for review, the IRO may either overturn or uphold it. If the claim is overturned, the health insurer is required to cover the health care services according to the terms of the health insurance policy. If the claim is upheld, the IRO agrees with the original decision and rules that the insurer does not need to cover the claim costs.

Table 9 shows that the Department received 393 requests for an independent review in 2024. About 85 percent of these requests are eligible for a review. This has been steadily growing over the past 10 years, with a considerable jump between 2023 and 2024 when the number of eligible requests increased by 85 percent.

Table 9. Requests for Independent Reviews by Eligibility: 2015 - 2024

Year	Not Eligible		Eligible		Total
	Count	Percent of Total	Count	Percent of Total	Count
2015	30	27.0%	81	73.0%	111
2016	55	35.0%	102	65.0%	157
2017	35	22.0%	124	78.0%	159
2018	55	25.5%	161	74.5%	216
2019	51	28.3%	129	71.7%	180
2020	42	26.1%	119	73.9%	161
2021	35	23.5%	114	76.5%	149
2022	39	22.2%	137	77.8%	176
2023	52	22.2%	181	77.8%	233
2024	58	14.8%	335	85.2%	393
Average	45.2	23.4%	148.3	76.6%	193.5

Data Source: Utah Insurance Department

The Department also tracks the reasons for eligible independent reviews. As shown in Table 10, in 2024, close to half of the independent reviews were requested due to contractual denials. Experimental/investigational and medical necessity each had approximately a quarter of the requests. The total number of independent reviews has been increasing since 2021. Each of the three categories increased between 2023 and 2024, with the highest absolute number increase in the experimental/investigational category.

Table 10. Requests for Independent Reviews by Reason: 2015 - 2024

Year	Contract Denial		Experimental/ Investigational		Medical Necessity		Total
	Count	Percent of Total	Count	Percent of Total	Count	Percent of Total	Unique Count ^a
2015	33	29.7%	32	28.8%	46	41.4%	111
2016	27	17.2%	42	26.8%	88	56.1%	157
2017	13	8.2%	41	25.8%	105	66.0%	159
2018	9	4.2%	37	17.1%	170	78.7%	216
2019	28	14.7%	40	20.9%	123	64.4%	180
2020	80	35.1%	25	11.0%	123	53.9%	161
2021	73	32.9%	35	15.8%	114	51.3%	149
2022	85	38.8%	48	21.9%	86	39.3%	176
2023	157	51.0%	75	24.4%	76	24.7%	233
2024	191	47.8%	111	27.8%	98	24.5%	393
Average	69.6	33.0%	48	22.7%	102	48.3%	211

Data Source: Utah Insurance Department

Note: Contract denials may include rescissions. Rescissions are rare and not broken out as a separate category.

^a An independent review may have more than one reason code, so totals may be higher than the actual number of independent reviews.

On average, about 76 percent of independent reviews are eligible for review. In 2024, 85 percent of requests were eligible. Of these, 47.8 percent were upheld, while 51.6 percent were overturned (Table 11). An additional 2 cases were partially overturned in 2024. These were included in the total, but not included in either of the other categories.

Table 11. Independent Review Decisions by Outcome: 2015 – 2024

Year	Upheld		Overturned		Total Eligible
	Count	Percent of Total	Count	Percent of Total	Count
2015	50	61.7%	31	38.3%	81
2016	52	51.0%	50	49.0%	102
2017	55	44.4%	69	55.6%	124
2018	72	44.7%	89	55.3%	161
2019	61	47.3%	68	52.7%	129
2020	55	46.2%	64	53.8%	119
2021	51	44.7%	63	55.3%	114
2022	65	47.4%	72	52.6%	137
2023	92	50.8%	89	49.2%	181
2024	160	47.8%	173	51.6%	335 ^a
Average	71.3	55.9%	76.8	60.2 %	148.3

Data Source: Utah Insurance Department

^a The total of upheld and overturned in 2024 do not add up to the total column because there were 2 partially overturned cases

Utah’s Comprehensive Health Insurance Market

Comprehensive health insurance made up 41.2 percent of the commercial health insurance market in the State of Utah (see Table 3) and covered just over 24 percent of Utahns (see Table 1) in 2024. It is the only type of major medical health benefit plan directly regulated by

the Insurance Department. The following analysis of the comprehensive health insurance market examines various aspects of the market including state of domicile, group size, health benefit plan types, and market trends.

Comprehensive Market by Domicile

The state of domicile is the state in which an insurer's home office is located. An insurer can only be domiciled in one state. Utah's domestic insurers are those that have a home office in Utah. These companies make up a larger share of the market than foreign insurers, which have their home offices in other states.

Approximately 86 percent of the comprehensive health insurance market in Utah is covered by 12 domestic insurers. Fifteen foreign insurers represent the remaining 14 percent. Premiums in Utah were higher for foreign insurers than domestic, with \$497 Per Member Per Month (PMPM) for foreign insurers and \$451 PMPM for domestic insurers. Loss ratios were higher for domestic insurers (see Table 13).

Table 12. Total Comprehensive Health Insurance Market by Domicile for 2024

Domicile	Insurer Count	Member Count	Direct Earned Premium	Market Share	Loss Ratio	Premium PMPM ^a
Domestic	12	742,958	\$3,987,793,472	86.4%	86.4	\$451
Foreign	15	104,697	\$627,195,740	13.6%	79.3	\$497
Total	27	847,655	\$4,614,989,212	100.0%	85.4	\$456

Data Source: Utah Accident & Health Survey

^a Direct earned premium per member per month

Comprehensive Health Insurance Market by Group Size

Comprehensive health insurance plans are sold either as an individual policy or a group policy. Individual policies are sold directly to individual consumers. In contrast, group policies are sold to an employer as a single contract. Groups with 1 to 50 eligible employees are classified as small employer groups. Groups with 51 or more eligible employees are classified as large employer groups. This is then further split into large group 51 to 100, and large group 101 or more.

Prior to the start of the Patient Protection and Affordable Care Act (ACA), individual and small group policy rates were based on the health status of the people enrolled, as required by state law. There were no federal regulations about how health insurers set their rates. Once the ACA went into effect, individual, small group, and large group comprehensive health insurance policies began using community rating. This means that rates are set so the risk is spread over the entire community of insured members, and individuals pay similar rates, no matter their health status.

Under the ACA, the only factors that may be used in setting rates are the number of individuals or family members enrolled, geographic area (some geographic areas have higher medical costs than others), age (older adults have higher health care costs than younger adults, but the top rating tier cannot be more than the 3 times the bottom tier), and tobacco use (rates for tobacco

users cannot be more than 1.5 times the rate of non-tobacco users). These changes mean that traditional rating factors like health status and gender are no longer used. These changes have the most impact on the individual market, where rates were mostly based on the health status of an individual.

In 2024, large group policies reported a higher PMPM premium (\$472) than individual (\$455) or small group (\$429) policies. Loss ratios were slightly higher for small and large group policies than for individual policies (see Table 13). Individual policies purchased through the Federally Facilitated Marketplace (FFM) may also receive premium subsidies through the Advance Premium Tax Credit (APTC). About 92 percent of the individual policies sold through the FFM received an APTC.

Table 13. Total Comprehensive Market by Group Size for 2024

Group Size	Insurer Count ^a	Member Count	Direct Earned Premium	Market Share	Loss Ratio	Premium PMPM ^b
Total Individual	16	405,818	\$2,172,112,259	47.1%	85.8	\$455
Small Group (1-50)	7	141,941	\$744,831,233	16.1%	86.1	\$429
Large Group (51+)	16	299,896	\$1,698,045,720	36.8%	84.3	\$472
Total Group	16	441,837	\$2,442,876,953	52.3%	84.9	\$458
Total Comprehensive	27	847,655	\$4,614,989,212	100.0%	85.4	\$456

Data Source: Utah Accident & Health Survey

^a Insurer count column does not add up to the total because an insurer may have more than one plan type.

^b Direct earned premium per member per month

Comprehensive Market by Plan Types

Comprehensive health insurance is classified into five major plan types: Fee for Service (FFS), Preferred Provider Organization (PPO), Health Maintenance Organization (HMO), Health Maintenance Organization with Point of Service features (HMO with POS), and Exclusive Provider Organization (EPO). These plan types differ in the amount of managed care used to maintain quality and manage the cost of healthcare services. The term “managed care” refers to how insurers monitor quality of care (such as disease management programs) and to reduce the utilization and cost of health care services (such as pharmacy benefit managers and medical review boards). HMO plans generally involve the most management of care, whereas FFS plans typically do not have any managed care.

FFS plans are traditional insurance plans. Under FFS plans, members can use any health care provider they choose (as long as the services are covered under the insurance policy). There are no preferred provider networks, and all services are reimbursed at the same cost sharing level (usually a fixed percentage of billed charges).

A PPO plan offers a network of “preferred” providers who have been contracted to provide services at reduced fees. People with this plan are encouraged to use these providers, as their costs are usually lower than other providers. Providers outside of the network can also provide services to people on these plans, but they may deny services or reimburse services at a lower rate. PPO plans usually include deductibles, copays, or coinsurance.

HMO plans provide services through a network of providers who have negotiated a fee schedule with the HMO. Members enrolled in the plan generally pay a deductible and a fixed copay for health care visits and drugs. Services are usually not available outside the provider network, except for emergencies.

HMO with POS plans are HMO plans that allow members to use providers outside the HMO network, but at a lower reimbursement rate. This results in members paying a much larger portion of the cost for health care services on top of the fixed copay and deductibles.

An EPO plan is similar to a PPO, offering a network of “preferred” providers that have contracted to provide health care services for a reduced fee. But unlike a PPO, members may not use providers outside of the network. EPO plans are similar to HMO plans in that services are usually limited to an exclusive set of network providers, except in cases of an emergency.

HMO, HMO with POS, PPO, and EPO plans are considered managed care plans. In 2024, 97 percent of Utah’s comprehensive health insurance market involved some type of managed care, with 72 percent in an HMO or an HMO with POS, as shown in Table 14.

Table 14. Total Comprehensive Market by Plan Type for 2024

Plan Type	Insurer Count ^a	Member Count	Direct Earned Premium	Market Share ^d	Loss Ratio	Premium PMPM ^b
Fee for Service	11	19,969	\$125,726,244	2.7%	84.1	\$492
Preferred Provider Organization	15	160,730	\$910,206,918	19.7%	81.5	\$477
Exclusive Provider Organization	3	30,853	\$242,629,305	5.3%	93.5	\$630
Health Maintenance Organization	6	468,166	\$2,399,938,783	52.0%	85.1	\$435
HMO with Point of Service features	2	167,937	\$936,487,962	20.3%	88.0	\$458
Total	27	847,655	\$4,614,989,212	100.0%	85.4	\$456

Data Source: Utah Accident & Health Survey

^a Insurer count column does not add up to the total because an insurer may have more than one plan type.

^b Direct earned premium per member per month

^c Percent may not add up to 100% due to rounding

Premium PMPM was higher for EPO plans compared to the other plan types, while HMO plans were the lowest among traditional insurance products. However, this comparison does not consider differences in plan structure, covered benefits, health status, or demographics. One reason some plans have lower premiums may be a higher deductible and fewer benefits. When a member chooses a higher deductible, the insurer covers fewer services, and the person is responsible for a greater share of their health care expenses. Because of this, the insurer has less financial risk, as shown by a lower premium. Another cost control measure used by insurers is the size of the provider network. Some plans have smaller networks, which limits the number of providers a person can use to receive covered services. The insurer uses smaller networks to negotiate with providers and drive more members to a small provider community. These narrow network plans result in lower provider reimbursements and lower member premiums.

Comprehensive Health Insurance Market by Regulatory Type

The federal government requires compliance with specific statutes for health insurers to provide

products to meet the needs of various populations. Tables 15-17 describe some of the regulatory types that have been created because of either state or federal legislation, and for which comprehensive health insurers have reported enrollment in Utah.

ACA Compliant Plans vs Non-ACA Compliant Plans. ACA compliant plans are those that are in full compliance with the federal regulations under the ACA. Non-ACA compliant plans are plans that have qualified for a partial exemption of the ACA regulations. These plans are called either grandfathered plans or transitional plans. In 2024, the majority of members (96 percent) of the overall comprehensive health insurance market were enrolled in ACA compliant plans (see Table 15), with 95 percent of the large and small group markets and 98 percent of the individual market enrolled in ACA compliant plans.

Off-Exchange Plans vs On-Exchange Plans. Plans can also be divided into “Off-Exchange” or “On-Exchange” plans. An Off-Exchange plan is one sold outside the federal exchange. In other words, they are sold directly to individuals and employer groups by the commercial health insurer without a health exchange. On-Exchange plans are sold on the FFM. All small and large group health benefit plans are Off-Exchange plans. Most (about 65 percent) of the comprehensive health insurance market were enrolled in Off-Exchange plans. The higher percentage of Off-Exchange plans is due to employer groups not having an exchange option. About 73 percent of the individual market were enrolled in the FFM. Off-Exchange membership was enrolled in both ACA compliant plans (94 percent) and Non-ACA complaint plans (6 percent).

Federally Facilitated Marketplace (FFM). The FFM is Utah’s health exchange for individuals. Policies sold through the FFM are rated using community rating and may be eligible for federal subsidies and income support for purchasing insurance. As of December 2024, there were 296,173 members (about 35 percent of the market) and 6 comprehensive health insurers participating in the FFM (see Table 15). All of the policies sold through the FFM are required to be ACA compliant plans.

Table 15. Total Comprehensive Market by ACA Market Segment for 2024

Market Segment by Group Size	Insurer Count ^a	Member Count	Percent of Members
Individual	16	405,818	47.9%
<i>Non-ACA Compliant</i>			
Off-Exchange	10	6,530	0.8%
<i>ACA Compliant</i>			
Off-Exchange	5	103,115	12.2%
Federally Facilitated Marketplace	6	296,173	34.9%
Small Group	8	141,941	16.8%
<i>Non-ACA Compliant</i>			
Off-Exchange	4	13,888	1.6%
<i>ACA Compliant</i>			
Off-Exchange	7	128,053	15.1%
Large Group	19	299,896	35.4%
<i>Non-ACA Compliant</i>			
Off-Exchange	7	10,371	1.2%

<i>ACA Compliant</i>			
Off-Exchange	16	289,525	34.2%
Total	27	847,655	100.0%
<i>Non-ACA Compliant</i>			
Off-Exchange	14	30,789	3.6%
<i>ACA Compliant</i>			
Off-Exchange	16	520,693	61.4%
Federally Facilitated Marketplace ^b	6	296,173	34.9%

Data Source: Utah Accident & Health Survey

Note: Estimates may not total exactly due to rounding. Data is current as of Dec. 31, 2024.

^a Insurer count column does not add up to total because an insurer may have more than one market segment.

Metal Tier Plans (Actuarial Value). ACA compliant plans can also be classified by actuarial value. This is a way to measure the relative cost-sharing value of plans. For example, a Gold plan covers approximately 80 percent of the eligible health care costs under the health benefit plan. The member is responsible for the rest. By comparison, a Bronze plan covers only about 60 percent of eligible health care costs, and the member is responsible for a higher portion of the cost. Starting in 2018, Bronze plans included an extended Bronze subcategory. This can include benefit options near the average actuarial value of 60 percent, but they can range from 56 percent to 65 percent. Plans with a higher actuarial value are usually more expensive, and those with a lower actuarial value are usually less expensive. However, the cost individuals pay can vary widely depending on their circumstances.

A majority of members on the FFM were enrolled in Silver plans (58 percent), followed by Bronze plans (40 percent), Gold plans (2 percent), and Platinum plans (less than 1 percent). In 2024, no catastrophic plans were offered in Utah. Under the ACA, catastrophic plans are only available in the individual market to individuals under the age of 30 or those with a hardship exemption (see Table 16).

Table 16. Metal Tier Plans on Federally Facilitated Marketplace for 2024

Market Segment by Metal Tier	Member Count	Percent of Members
Federally Facilitated Marketplace	296,173	100.0%
Platinum (90% AV)	1,018	0.3%
Gold (80% AV)	6,067	2.1%
Silver (70% AV)	171,439	57.9%
Bronze (60% AV)	117,649	39.7%

Data Source: Utah Accident & Health Survey

Note: Estimates may not total exactly due to rounding. Data is current as of Dec. 31, 2024. There were 6 commercial health insurers participating in the Federally Facilitated Marketplace as of Dec. 31, 2024.

HSA-Qualified High Deductible Health Plans . These are high deductible plans that can be combined with a savings account called a Health Savings Account (HSA). Federal law sets the minimum deductible on these plans. They also need to comply with federal guidelines to qualify for the HSA. Payments to an HSA are tax deductible and can be used to pay for current health care expenses or saved for the future. Once the health care expenses reach the level of the deductible, this plan will pay for all additional covered health care costs. High deductible health plans can also be used in conjunction with Health Reimbursement Arrangements (HRA). HRAs

are similar to HSAs, except the employer owns the savings account (rather than the individual) and only the employer can deposit funds into the account. In 2024, 230,296 members (27 percent of the market) were enrolled in HSA-qualified high-deductible plans (see Table 17).

Standard Plans. Standard plans do not qualify for HSA-Qualified High Deductible Health Plans. Most plans in Utah’s health insurance market are Standard Plans. There were 617,359 members (73 percent of the market) enrolled in Standard Plans in 2024 (see Table 18).

Table 17. HSA-Qualified High Deductible Plans for 2024

Market Segment by Group Size	Member Count	Percent of Members
Individual	405,818	47.9
HSA-Qualified High Deductible Health Plan	25,388	3.0%
Standard Plan	380,430	44.9%
Small Group	141,941	16.8%
HSA-Qualified High Deductible Health Plan	61,229	7.2%
Standard Plan	80,712	9.5%
Large Group	299,896	35.4%
HSA-Qualified High Deductible Health Plan	143,679	16.9%
Standard Plan	156,217	28.5%
Total	847,655	100.0%
HSA-Qualified High Deductible Health Plan	230,296	27.2%
Standard Plan	617,359	72.8%

Data Source: Utah Accident & Health Survey
Note: Estimates may not total exactly due to rounding. Data is current as of Dec. 31, 2024.

Comprehensive Health Insurance Market Trends

This section reports on four trends in Utah’s comprehensive health insurance market: the number of insurers, the number of insured members, insurance costs, and the market's financial status. Each measure looks at a different aspect of the market’s “health.”

Number of Insurers. The Department tracks the number of commercial health insurance companies that offer comprehensive health insurance. Table 18 shows that the overall number of comprehensive health insurers declined from 2015 to 2024. In 2024, the number of comprehensive health insurers in Utah has remained steady the last three years at 27, with an overall decrease of 12 compared to 10 years ago.

Table 18. Changes in the Number of Comprehensive Health Insurers: 2015 - 2024

Insurer Category	2015	2016	2017	2018	2019	2020	2021	2022	2023	2024	10 yr Net Change
Domestic Insurers											
Greater than 100 Million	4	4	3	3	3	3	3	3	2	4	0
Between 10 and 100 Million	4	4	5	3	1	3	3	4	4	2	-2
Between 1 and 10 Million	3	5	3	4	6	4	4	2	4	4	1
Less than 1 Million	1	0	1	1	1	1	2	2	1	2	1
Total Domestic	12	13	12	11	11	11	12	11	11	12	0

Foreign Insurers

Insurer Category	2015	2016	2017	2018	2019	2020	2021	2022	2023	2024	10 yr Net Change
Greater than 100 Million	1	1	1	1	1	2	2	3	3	3	2
Between 10 and 100 Million	5	5	5	4	4	3	3	2	2	1	-4
Between 1 and 10 Million	3	2	2	2	2	2	3	2	2	3	0
Less than 1 Million	18	17	17	13	10	9	9	9	9	8	-10
Total Foreign	27	25	25	20	17	16	17	16	16	15	-12
All Insurers											
Greater than 100 Million	5	5	4	4	4	5	5	6	5	7	2
Between 10 and 100 Million	9	9	10	7	5	6	6	6	6	3	-6
Between 1 and 10 Million	6	7	5	6	8	6	7	4	6	7	1
Less than 1 Million	19	17	18	14	11	10	11	11	10	10	-9
Total Utah	39	38	37	31	28	27	29	27	27	27	-12

Data Source: Utah Accident & Health Survey

Note: Comprehensive health insurers are counted by relative size, broken into four categories of direct earned premium measured in millions of US dollars.

The typical health insurer must be large enough to drive people to specific providers in order to remain competitive. While there is no absolute rule for how large an insurer must be, an insurer with a large membership base has greater leverage in contract negotiations with providers. This arrangement can benefit both consumers and providers. Consumers may benefit from lower prices and providers may benefit from more clients. Many small comprehensive health insurers cannot “drive volume” as effectively as a large insurer.

Most of the decline in the number of health insurers over the past 10 years has been among very small comprehensive health insurers, particularly foreign insurers with less than \$1 million in comprehensive health insurance premiums (see Table 18). In many cases, these very small companies are providing coverage for “non-situated” policies. These policies have been issued in another state to an employer with less than 25 percent of their employees living in Utah. The premium is reported as covering a Utah resident, but the policy itself was not sold in Utah. Many of these companies are not actively selling health insurance in the Utah health insurance market and are only here because they sold a health insurance policy to a company that has an employee who is currently a resident in the state. As a result, many of these insurers leave the market when the employees leave the company. Since these companies cover this specific type of person, they are likely not competing directly in the mainstream health insurance market in Utah.

Medium sized insurers have seen more fluctuation over the past decade. From 2015 to 2017, there was some turnover in the total number of medium insurers (between \$10 to \$100 million in premiums), and several new medium sized insurers entered the market, including several new domestic insurers that entered the market to participate in the FFM. In 2018 and 2019, the number of medium sized insurers declined as the market shifted, with several insurers decreasing in size. The number of medium sized insurers increased in 2020 and remained stable until 2024, when medium sized companies for both domestic and foreign insurers dropped to a total of 3 companies.

Large health insurance companies represent a majority of the comprehensive health insurance market. Large insurers account for 60 percent of the market. They create strength, stability, and choice for Utah’s comprehensive health insurance market.

Number of Members. Since 2015, Utahns insured by comprehensive health insurance as a relative percentage of Utah's total population has declined by 3.4 percent (Table 19). Comprehensive membership has averaged about 766,500 members over the last ten years (approximately 24 percent of Utah's population in any given year). In 2024, comprehensive membership increased by 7.2 percent compared to 2023. This increase appears to be due to changes in the large group market, which increased approximately 90,000 between 2023 and 2024.

Table 19. Changes in Comprehensive Health Insurance Membership by Group Size: 2015 - 2024

	2015	2016	2017	2018	2019	2020	2021	2022	2023	2024	<i>Net Change^a</i>
Individual	226,927	238,637	205,992	206,222	210,215	217,524	253,344	277,955	341,296	405,818	+178,891
Percent of population ^b	7.6%	7.8%	6.6%	6.5%	6.6%	6.6%	7.6%	8.2%	10.0%	11.6%	+4.0%
Small Group	192,306	177,948	173,004	161,316	155,776	155,963	161,722	164,251	153,681	141,941	-50,365
Percent of population	6.4%	5.8%	5.6%	5.1%	4.9%	4.8%	4.8%	4.9%	4.5%	4.1%	-2.4%
Large Group	406,876	375,818	375,322	354,309	346,556	332,988	329,380	325,931	295,711	299,896	-106,980
Percent of population	13.6%	12.3%	12.1%	11.2%	10.8%	10.2%	9.9%	9.6%	8.6%	8.6%	-5.0%
Total Group	599,182	553,766	548,326	515,625	502,332	488,951	491,102	490,182	449,392	441,837	-157,345
Percent of population	20.0%	18.1%	17.7%	16.3%	15.7%	14.9%	14.7%	14.5%	13.2	12.6%	-7.4%
Total Comprehensive	826,109	792,403	754,318	721,847	712,547	706,475	744,446	768,137	790,688	847,655	+21,546
Percent of population	27.6%	26.0%	24.3%	22.8%	22.2%	21.6%	22.3%	22.7%	23.1%	24.2%	-3.4%
Utah Population	2,995,919	3,051,217	3,101,833	3,161,105	3,205,958	3,271,616	3,337,975	3,380,800	3,417,734	3,506,838	+510,919

Data Sources: Utah Accident & Health Survey, Utah Population Estimates Committee, and the U.S. Census Bureau.

Note: Estimates may not add up exactly to totals due to rounding.

^a “Net Change” measures the difference in the absolute number of members from 2015 to 2024 as well as the change in membership as a relative percentage of Utah’s total population.

^b “Percent of population” estimates the membership as a relative percentage of Utah’s total estimated population in each particular year.

Table 20. Changes in Comprehensive Membership by Plan Type: 2015 - 2024

Plan Type ^a	2015	2016	2017	2018	2019	2020	2021	2022	2023	2024	10 yr Net Change ^b
FFS	15,018	21,621	20,051	19,863	21,049	21,890	19,912	19,396	26,605	19,969	+4,951
Percent of population ^c	0.5%	0.7%	0.6%	0.6%	0.7%	0.7%	0.6%	0.6%	0.8%	0.6%	+0.1%
PPO	248,071	234,642	237,760	197,909	195,821	180,830	170,891	169,020	156,521	160,730	-87,341
Percent of population	8.3%	7.7%	7.7%	6.3%	6.1%	5.5%	5.1%	5.0%	4.6%	4.6%	-3.7%
EPO	-	-	4,052	5,738	24,590	22,977	25,355	25,226	30,743	30,853	+26,801
Percent of population	-	-	0.10 %	.20%	0.80%	0.70%	0.8%	0.8%	0.9%	0.9%	+0.8%
HMO	267,842	294,663	268,340	265,380	^d 270,379	278,237	347,315	369,732	409,252	468,166	+200,324
Percent of population	8.9%	9.7%	8.7%	8.4%	8.4%	8.5%	10.4%	10.9%	12.0%	13.4%	+4.5%
HMO with POS	295,178	237,425	223,029	214,105	202,321	200,163	181,102	183,437	167,567	167,937	-127,241
Percent of population	9.9%	7.8%	7.2%	6.8%	6.3%	6.1%	5.4%	5.4%	4.9%	4.8%	-5.1%
Total Comprehensive	826,109	792,403	754,318	721,847	712,547	706,475	744,446	768,137	790,668	847,655	+21,546
Percent of population	27.6%	26.0%	24.3%	22.8%	22.2%	21.6%	22.3%	22.7%	23.1%	24.2%	-3.4%
Utah Population	2,995,919	3,051,217	3,101,833	3,161,105	3,205,958	3,271,616	3,337,975	3,380,800	3,417,734	3,506,838	+510,919

Data Sources: Utah Accident & Survey, Utah Population Estimates Committee, and the U.S. Census Bureau.

Note: Estimates may not add up exactly to totals due to rounding. Estimate totals may differ from previous reports due to category changes.

^a Plan Types Key: FFS = Fee For Service / Indemnity, PPO = Preferred Provider Organization, EPO = Exclusive Provider Organization, HMO = Health Maintenance Organization, HMO with POS = Health Maintenance Organization with Point of Service features

^b “Net Change” measures the difference in the absolute number of members from 2015 to 2024 as well as the change in membership as a relative percentage of Utah’s total population.

^c “Percent of population” measures the plan membership as a relative percentage of Utah’s total population in each particular year.

The number of members in PPO plans and HMO with POS plans have decreased and the number of members in EPO and HMO plans increased. The increase in HMO plans appears to be due to a shift from HMO with POS plans to HMO plans offered in the market. FFS plans have fluctuated during that same period (Table 20).

Membership in government sponsored plans in Utah continues to show a steady increase (see Table 21) until 2023. In March, 2020, the federal government passed the Families First Coronavirus Response Act. This law increased Medicaid enrollment by providing enhanced federal funding for continuous enrollment and allowing individuals to remain enrolled during the COVID-19 public health emergency. However, Medicaid began the process of returning to normal operations in 2023, causing membership to decline in 2023 and 2024. Medicaid enrollment declined about 31 percent from its peak in 2022 through the end of 2024, while Medicare and CHIP membership increased.

Table 21. Changes in Government Sponsored Health Benefit Plans: 2015-2024

Plan Type	2015	2016	2017	2018	2019	2020	2021	2022	2023	2024	Net Change ^a
Medicare	340,968	355,492	371,770	398,399	424,323	410,689	428,379	439,708	451,986	465,601	124,633
Medicaid	295,123	297,552	298,251	303,913	317,353	366,428	455,927	492,316	456,286	337,462	18,026
CHIP	16,588	18,577	19,651	18,959	17,512	16,354	9,521	6,473	7,859	12,512	-4,076
Government Plans	665,882	688,925	703,277	734,493	759,188	793,471	893,827	938,497	916,131	815,575	149,693
As percent of population^b	22.2%	22.6%	22.7%	23.2%	23.7%	24.4%	26.8%	27.8%	26.8%	23.3%	1.1%

Data Sources: Centers for Medicare and Medicaid Services, Utah Department of Health and Human Services

Note: Estimates may not total exactly due to rounding.

^a “Net Change” measures the difference in the absolute number of members from 2015 to 2025 as well as the change in membership as a relative percentage of Utah’s total population.

^b “Percent of population” measures the relative percentage of Utah’s total population in each particular year.

Cost of Insurance. Utah’s comprehensive health insurance premiums have increased over the last 10 years. From 2015 to 2025, the average premium PMPM for comprehensive health insurance has increased an average of 7.5 percent per year. In 2024, premium growth increased and the average premium PMPM for comprehensive health insurance was 6.3 percent higher than in 2023 (see Table 22).

Table 22. Comprehensive Premium Compared to National Economic Trends: 2015 – 2024

Year	Comprehensive Premium in Utah				National Economic Trends
	Total Premium ^a	Premium PMPM ^b	Premium PMPY ^c	Annual % Change	Health Insurance Premium Annual Percent Change ^d
2015	\$2,767,877,369	\$280	\$3,360	1.1%	4.2%
2016	\$2,929,832,909	\$300	\$3,600	7.1%	3.4%
2017	\$3,020,205,133	\$330	\$3,960	10.0%	3.4%
2018	\$3,325,579,764	\$379	\$4,548	14.8% ^e	4.5%
2019	\$3,287,778,900	\$383	\$4,596	1.1%	4.9%
2020	\$3,295,470,583	\$386	\$4,632	0.8%	3.7%
2021	\$3,392,957,126	\$388	\$4,656	0.5%	4.1%
2022	\$3,690,309,784	\$404	\$4,848	4.1%	1.1%
2023	\$4,038,507,570	\$429	\$5,148	6.2%	6.6%

Year	Comprehensive Premium in Utah				National Economic Trends
	Total Premium ^a	Premium PMPM ^b	Premium PMPY ^c	Annual % Change	Health Insurance Premium Annual Percent Change ^d
2024	\$4,614,989,212	\$456	\$5,472	6.3%	6.7%

Data Sources: Utah premium data are from the Utah Accident & Health Survey from 2015 to 2024. The national trend data used as a comparison comes from the 2022 Kaiser Family Foundation Employer Health Benefits Survey.

^a Total direct earned premium

^b Direct earned premium per member per month

^c Direct earned premium per member per year

^d “Health Insurance Premium” trends are based on premium changes for family coverage under an employer based plan.

One of the main causes of higher premiums is the increasing cost of health care. From 2015 to 2024, the average losses PMPM for comprehensive health insurance have increased by about 3.6 percent per year. In 2024, the average losses PMPM for comprehensive health insurance were 4.6 percent higher than in 2023 (see Table 23).

Health care costs are driven by many factors. The main factors include changes in medical technology, pharmaceutical costs, government regulations, payment models, demographics, lifestyle choices, and general inflation (PriceWaterhouseCoopers, 2018). Utilization of healthcare services and unit prices are also important (PriceWaterhouseCoopers, 2017). Other studies have found evidence of excess spending on unnecessary medical services, administrative costs and complexity, fraud and abuse, and service pricing (Institute of Medicine, 2012; Shrank, Rogstad, and Parekh, 2019). Coverage expansions under the ACA and increases in retail prescription drug costs have also affected the cost of health care (Hartman, Martin, Espinosa, Catlin, and the National Health Expenditure Accounts Team, 2018). Prescription drug spending is growing faster than other types of health care spending (American Academy of Actuaries, 2018). Additionally, increases in the price of health care, particularly the price of prescription drugs, has become a key area of focus as a way to manage rising healthcare costs (PriceWaterhouseCoopers, 2017; PriceWaterhouseCoopers, 2018).

Table 23. Comprehensive Losses Compared to National Health Care Spending: 2015 - 2024

Year	Comprehensive Losses in Utah				National Health Care Expenditures (in Millions of Dollars)			
	Loss Ratio ^a	Losses PMPM	Losses PMPY	Annual Percent Change	Total NHE (all sources)	Annual Percent Change	NHE Private Health Insurance	Annual Percent Change
2015	95.3	\$267	\$3,204	9.4%	\$3,165,394	5.4%	\$976,911	5.8%
2016	92.9	\$279	\$3,348	4.5%	\$3,307,404	4.5%	\$1,030,833	5.5%
2017	89.1	\$294	\$3,528	5.4%	\$3,446,454	4.2%	\$1,080,044	4.8%
2018	83.1	\$315	\$3,780	7.1%	\$3,604,428	4.6%	\$1,129,836	4.6%
2019	84.4	\$323	\$3,876	2.5%	\$3,757,382	4.2%	\$1,157,796	2.5%
2020	83.1	\$321	\$3,852	-0.6%	\$4,144,077	10.3%	\$1,145,190	-1.1%
2021	86.7	\$337	\$4,044	5.0%	\$4,255,127	2.7%	\$1,211,431	5.8%
2022	86.4	\$349	\$4,188	3.6%	\$4,525,845	6.4%	\$1,313,790	8.4%
2023	86.6	\$373	\$4,476	6.9%	\$4,866,494	7.5%	\$1,464,648	11.5%
2024	85.6	\$390	\$4,680	4.6%	\$5,263,325	8.2%	\$1,616,841	10.4%

Data Sources: Utah loss data are from the Utah Accident & Health Survey from 2015 to 2024. The National Health Care Expenditure data are from the Centers for Medicare and Medicaid Services (CMS), Office of the Actuary (2022). NHE historical data were used for 2015 to 2024. NHE projected data were used for 2024.

^a Ratio of direct incurred losses to direct earned premium

^b Direct incurred losses per member per month

^c Direct incurred losses per member per year

Estimates based on national health expenditure data suggest that national health care spending is expected to grow by 5.5 percent per year between 2024 and 2033. Enrollment in health insurance coverage and health care spending are projected to grow over the next decade (Keehan, Fiore, Poisal, Cuckler, Sisko, Smith, Madison, and Rennie, 2023).

A measurement of the current financial condition of the market shows the cost of health care continues to create significant economic pressure on comprehensive health insurers. Some of the big drivers for increasing costs for comprehensive insurance companies include increased costs for medical services, higher utilization of services, and more expensive prescription drugs. As a result, comprehensive insurers must raise premiums to cover their costs.

In addition to claim costs, comprehensive insurers must pay general administrative costs such as general business expenses and the cost of processing claims. Commercial health insurers are required to maintain adequate financial reserves to remain financially solvent, reinvest in new equipment, invest in new markets, and buy and maintain needed capital.

For Utahns, higher premiums mean that health care continues to be expensive. For a single individual, the average premium per member increased by 115 percent (\$3,084 PMPY) over 10 years (excluding any advance premium tax credits the individual may have received). Both consumers and employers are impacted by these increases. In most cases, employers pay a significant portion of this premium. Nationally, employers pay more than two-thirds of the premium cost (Kaiser Family Foundation, 2023a; Kaiser Family Foundation, 2023b). However, many employers are responding to the rising cost of health care by increasing the employee's portion of the premium, reducing benefits, increasing deductibles and cost sharing, or looking at new plan designs to reduce costs (Kaiser Family Foundation, 2023a). Similar changes are also taking place in the individual market. These changes are hard for many people to manage because wage increases have not kept pace with health care costs. As a result, many consumers struggle with the cost of insurance and being able to receive necessary health care (see Table 24).

Table 24. Changes in Comprehensive Premium and Per Capita Income: 2015 - 2024

	2015	2016	2017	2018	2019	2022	2021	2022	2023	2024
Premium PMPY ^a	\$3,360	\$3,600	\$3,960	\$4,548	\$4,596	\$4,632	\$4,656	\$4,848	\$5,172	\$5,472
Percent change in Premium	1.1%	7.1%	10.0%	14.8%	1.1%	0.8%	0.5%	4.1%	6.7%	5.8%
Per Capita Income in Utah	\$40,459	\$41,750	\$43,241	\$45,665	\$48,580	\$52,220	\$56,019	\$59,457	\$63,700	\$66,515 ^b
Percent change in Income	6.0%	3.2%	3.6%	5.6%	6.4%	7.5%	7.3%	6.1%	5.6%	4.4%

Data Sources: Utah premium data are from the Utah Accident & Health Survey. Per capita income data are from the 2024 Economic Report to the Governor, David Eccles School of Business and the Utah Governor's Office of Planning and Budget.

^a Direct earned premium per member per year

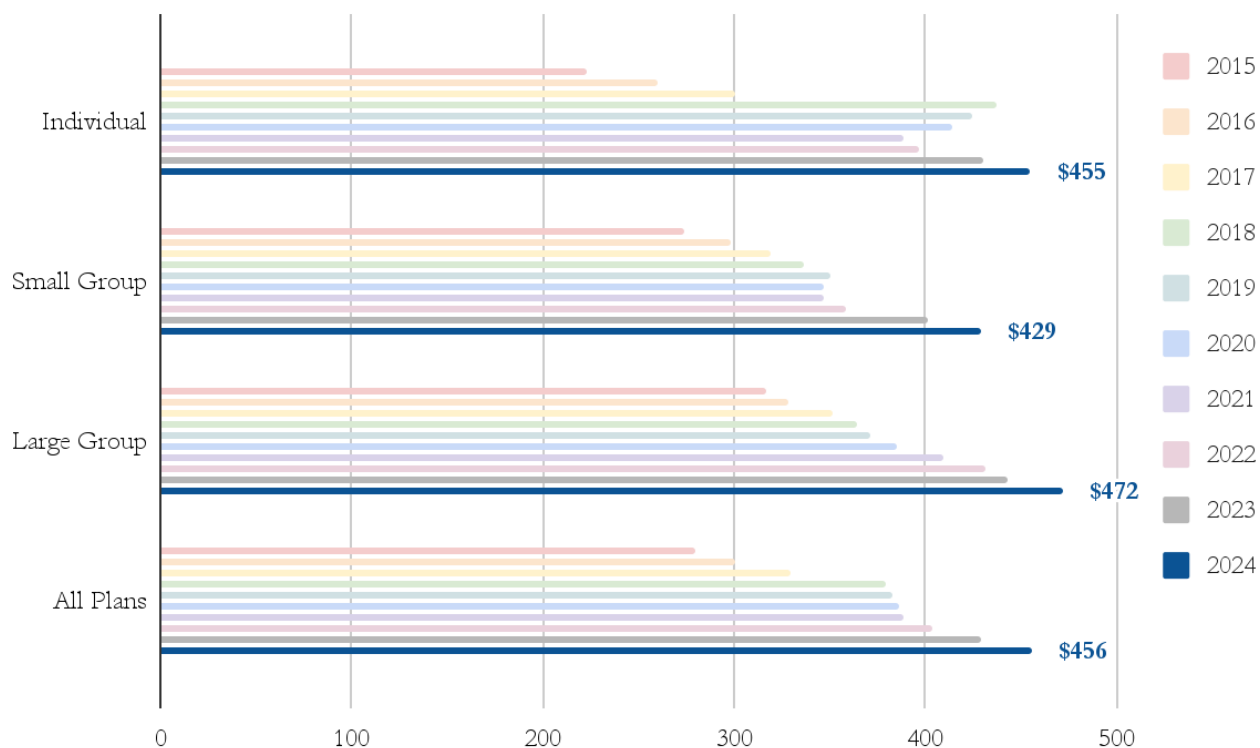
^b Estimate

Under the ACA, policies are created using community rating. This means that the insurance risk is spread over the entire community of insured members, regardless of their health status. Because of this, the cost of covering higher risk and lower risk individuals tends to average out. This method is more beneficial to individuals with higher health care costs. Starting in 2014, the individual market began using a form of community rating to set rates, including coverage for individuals with higher costs. This has contributed to increased insurance rates. Compared to 2023, individual premium PMPM rates increased in 2024 by 5.8 percent, small group premium

PMPM increased by 7 percent, and large group premium PMPM increased by 6.6 percent (see Figure 2).

In the past, increases in large group plan premiums have had the greatest impact on market premium trends. More Utahns in the comprehensive health insurance market are covered by large group plans than by any other type, resulting in a larger impact on market averages than changes in the individual or small group markets.

Figure 2. Comprehensive Premium PMPM by Group Size: 2015 - 2024



Data Source: Utah Accident & Health Survey

Although Utah continues to see increases in the cost of comprehensive health insurance, Utah premiums appear to be lower than the national average, according to the NAIC (see Table 25). For example, in 2024, the average premium for Utah's comprehensive health insurers was \$456 PMPM. However, the average premium for insurance companies reporting to the NAIC was \$525 PMPM. Although this comparison does not account for differences in benefits, health status, or demographics, this data suggests that Utah's average premium is lower than the average premium reported.

The premiums that consumers actually pay may differ significantly from the market average depending on their individual circumstances and plan choice. Although Utah's premiums may be lower by this measure, Utah's premiums in 2024 increased at higher rates than comprehensive health insurers nationally (6.3 percent for Utah, 0.2 percent for comprehensive health insurers reporting to NAIC), and continue to be financially challenging for many consumers.

Table 25. Comparison of Utah Premium to National Premium: 2015 - 2024

Year	Utah Estimate		National Estimate	
	Premium PMPM for Comprehensive Health Insurance ^a	Annual Percent Change	Premium PMPM for Comprehensive Health Insurance	Annual Percent Change
2015	\$280	1.1%	\$364	4.6%
2016	\$300	7.1%	\$389	6.9%
2017	\$330	10.0%	\$423	8.7%
2018	\$379	14.8%	\$461	9.0%
2019	\$383	1.1%	\$467	1.3%
2020	\$386	0.8%	\$486	4.1%
2021	\$388	0.5%	\$488	0.4%
2022	\$404	4.1%	\$495	1.4%
2023	\$429	6.2%	\$524	5.9%
2024	\$456	6.3%	\$525	0.2%
10-year Average	\$374	9.3%	\$462	2.0%

Source: Utah Accident & Health Survey and the NAIC Financial Database

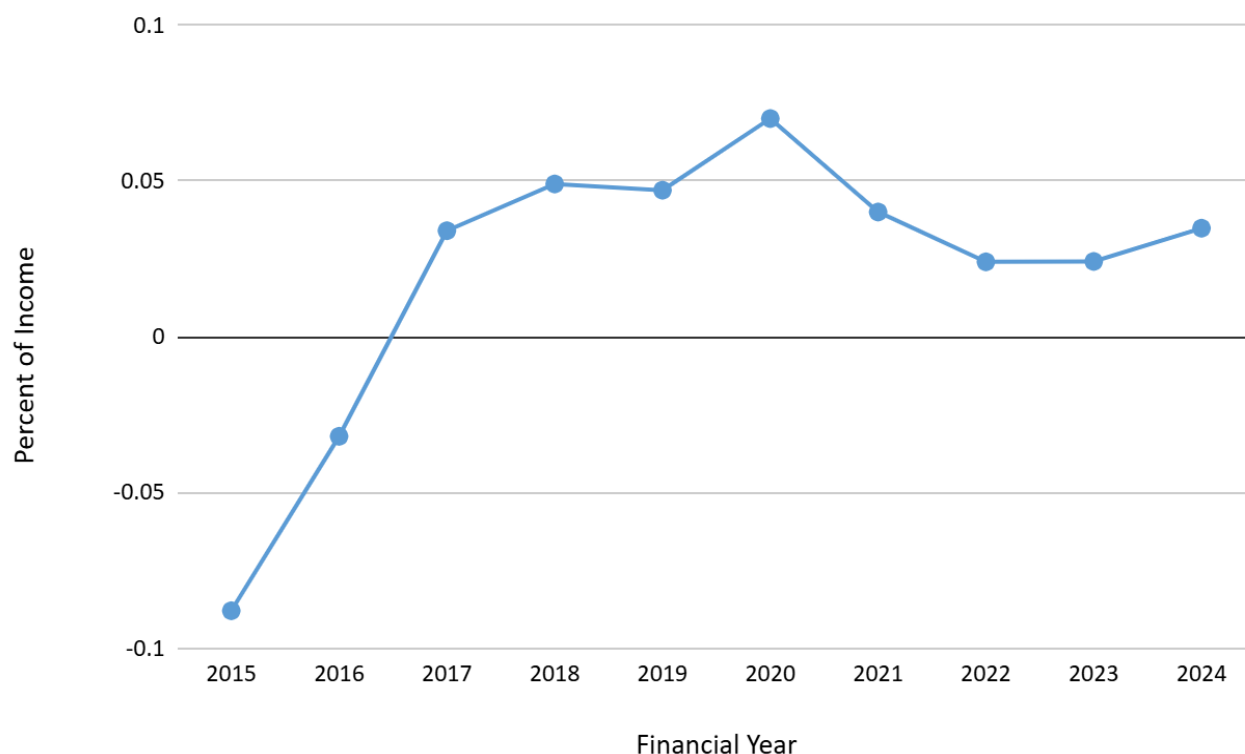
Note: The Utah estimate is based on data obtained from the Utah Accident & Health Survey for comprehensive health insurance. The national estimate is based on data obtained from the NAIC Financial Database. The data represents the average premium per member per month for comprehensive health insurance business as reported by commercial health insurers who filed on the annual financial statement for health related insurance business. Both data sources include only information on commercial health insurers.

^a Premium per member per month is the average premium per person per month for comprehensive health insurance. This is the estimated cost of health insurance for all types of hospital and medical coverage on a per person basis. A division into single and family rates is not possible using data from the Utah Accident & Health Survey or the NAIC Financial Database. This comparison does not control for differences in plan structure, covered benefits, health status, or demographics.

Financial Status of the Market. To measure the current financial condition of the market, the financial results of the major comprehensive health insurers in Utah were used as an index of Utah's comprehensive health insurance market. These companies were selected because: 1) they represent more than 80 percent of the comprehensive health insurance market, 2) a majority of their revenues come from Utah businesses, and 3) their business model is that of a comprehensive health insurer. These companies are Utah's best examples of comprehensive health insurers and they can provide an index of how well comprehensive health insurers are doing in the Utah market over time (see Figure 3). The results of this index indicate that Utah's comprehensive health insurance market has experienced an average financial gain of 2 percent in net income per year for the last 10 years. In 2024, these companies reported an average gain in net income of 3.5 percent.

As shown in Figure 3, income increased between 2015 and 2020. This was likely due to the federal government ending the federal funding for the Cost Share Reduction (CSR) payment program. This required comprehensive health insurers to raise rates higher than they would have been had the CSR payments continued. This led to a financial recovery. Since 2020, the percent of income has remained between 0 and .5 percent (see Figure 3).

Figure 3. Income After Expenses for Comprehensive Health Insurers: 2015 - 2024



Data Source: NAIC Annual Financial Statements

Note: This figure represents the ratio of net income to total revenue as reported on the NAIC annual statement for the major managed care health insurers that have been operating in Utah from 2015 to 2024. Results are rounded to the nearest 0.1%.

Health Insurance Mandates. The ACA requires an individual comprehensive health insurance plan sold on the FFM to meet a minimum standard, or essential health benefits requirement. If a state mandates an insurer to offer a benefit that exceeds the essential health benefits, the state must reimburse those costs through a defrayal payment (45 C.F.R. 155.170). In 2019, SB95 was passed, which required a comprehensive health insurer in the individual or large group market to provide coverage for behavioral health treatment for a person with an autism spectrum disorder (Utah Code § 31A-22-642(5)(b)). The mandated coverage in the individual market is subject to the federal defrayal payment requirement (see Table 26). The state defrayal payments increased by 49 percent between 2023 and 2024 and are expected to continue rising.

Table 26. State Benefit Mandate Defrayal Payments for Autism Behavioral Health Treatment: 2020 - 2024

Year	Member Months ^a	Defrayal Cost PMPM	State Defrayal Payments
2020	1,685	\$1,253	\$1,828,871
2021	2,362	\$2,159	\$4,630,581
2022	2,649	\$3,206	\$8,082,107
2023	3,904	\$3,272	\$12,209,957
2024	5,254	\$3,574	\$18,226,710

Data Source: Utah Insurance Department

^a The total months of coverage for members that received autism benefits during the year.

Health Care Preauthorization Reporting

Utah Code § 31A-22-650 requires insurance companies that have a health care preauthorization requirement to submit a report each year. Each insurer must report the percentage of authorizations for which they notified a provider of an authorization or adverse preauthorization determination more than one week after receiving the authorization request. This does not include claims involving urgent or emergency care as defined in 29 C.F.R. Sec. 2560.503-1. Some insurers may not have a preauthorization requirement for emergency health care as described in Utah Code § 31A-22-627.

The 27 comprehensive health insurers operating in Utah in 2024 each had a preauthorization requirement. Table 27 shows the preauthorization information reported to the Department in 2024. Sixteen companies reported authorizations processed more than one week after the day the authorization request was received. The insurer average was 9.7 percent.

Table 27. Healthcare Preauthorizations Processed After One Week in 2024

Insurer Name	State of Domicile	Percentage Processed After One Week
4 Ever Life Insurance Company	IL	88.0%
Aetna Health of Utah Inc.	UT	13.3%
Aetna Life Insurance Company	CT	5.3%
All Savers Insurance Company	IN	0.0%
American National Life Insurance Company of Texas	TX	0.0%
Balanced Insurance Company of Utah	UT	6.3%
Bridgespan Health Company	UT	6.4%
Connecticut General Life Insurance Company	CT	0.0%
Cigna Health and Life Insurance Company	CT	6.5%
Educators Health Plans Life, Accident & Health, Inc.	UT	0.0%
Equitable Financial Life Insurance Company	NY	0.0%
Health Care Service Corporation, a Mutual Legal Reserve Company	IL	5.9%
Humana Insurance Company	WI	0.0%
Imperial Health Plan of the Southwest, Inc.	UT	7.1%
Metropolitan Life Insurance Company	NY	0.0%
Molina Healthcare of Utah	UT	62.7%
MotivHealth Insurance Company	UT	8.2%
The Prudential Insurance Company of America	NJ	0.0%
Regence BlueCross BlueShield of Utah	UT	7.8%
SelectHealth, Inc.	UT	9.6%
Standard Life and Accident Insurance Company	TX	0.0%
State Farm Mutual Automobile Insurance Company	IL	0.0%
Transamerica Life Insurance Company	IA	0.0%
UnitedHealthcare Insurance Company	CT	4.6%
UnitedHealthcare of Utah, Inc.	UT	7.0%
University of Utah Health Insurance Plans	UT	30.7%
WMI Mutual Insurance Company	UT	33.3%
All Comprehensive Health Insurers	27	9.7%

Data Source: Utah Adverse Preauthorization Determination Survey

Utah's Stop-Loss Insurance Market

Stop-loss insurance protects against unexpected or catastrophic claims. Stop-loss insurance made up 3.5 percent of the commercial health insurance market in Utah in 2024 (see Table 3). This section focuses on medical stop-loss insurance. It provides insurance coverage for employer sponsored self-funded plans and is sold as an accident & health product in Utah. This analysis looks at market trends, state of domicile, group size, and coverage attachment points for the medical stop-loss market.

Stop-Loss Insurance Market Trends

Under the ACA, commercial and self-funded health benefit plans may not have annual or lifetime limits on essential health care benefits, which can increase the risk for these plans. Since the full implementation of the ACA, there appears to be increased demand from self-funded employers seeking ways to manage risk and health care costs. From 2023 to 2024, the number of members covered by stop-loss insurance decreased by 10 percent (see Table 28).

Table 28. Total Stop-Loss Market: 2015 - 2024

Year	Insurer Count	Member Count	Direct Earned Premium	Loss Ratio
2015	41	468,760	\$140,070,917	71.9
2016	44	607,058	\$171,862,070	82.9
2017	46	625,174	\$205,785,395	79.8
2018	48	591,099	\$241,941,465	80.0
2019	46	649,783	\$288,558,315	83.0
2020	42	663,501	\$311,042,258	72.5
2021	43	691,963	\$350,273,879	83.0
2022	46	720,377	\$380,021,434	89.0
2023	44	726,017	\$391,938,484	92.6
2024	40	648,749	\$391,125,926	101.3
Average	44	639,248	\$287,262,014	83.6

Data Source: Utah Accident & Health Survey

Stop-Loss Insurance Market by Domicile

Approximately 72 percent of the stop-loss insurance market was served by 35 foreign insurers, with 5 domestic insurers covering the remaining 28 percent of the market. Premiums were higher for domestic insurers than foreign insurers with \$73 PMPM for domestic insurers and \$56 PMPM for foreign insurers. Loss ratios were also higher for domestic insurers (see Table 29). Loss ratios were higher in 2024 than in 2023, which is consistent with the higher healthcare spending reported by third-party payers during this period.

Table 29. Total Stop-Loss Market by Domicile for 2024

Domicile	Insurer Count	Member Count	Direct Earned Premium	Market Share	Loss Ratio	Premium PMPM ^a
Domestic	5	126,417	\$109,837,943	28.1%	108.0	\$73
Foreign	35	522,332	\$281,287,983	71.9%	98.8	\$56

Domicile	Insurer Count	Member Count	Direct Earned Premium	Market Share	Loss Ratio	Premium PMPM ^a
Total	40	648,749	\$391,125,926	100.0%	101.3	\$60

Data Source: Utah Accident & Health Survey

Note: Estimates may not total exactly due to rounding.

^a Direct earned premium PMPM

Stop-Loss Insurance Market by Group Size

Stop-loss insurance plans are sold to employers with self-funded plans. Some employer sponsored self-funded plans purchase stop-loss insurance plans with lower attachment points, the point where an insurer starts to pay for a loss, to reduce their financial risk. Data was collected for three group sizes: Small Group (1 to 50 eligible employees), Large Group (51 to 100 eligible employees), and Large Group (101 or more eligible employees).

Table 30. Total Stop-Loss Market by Group Size for 2024

Group Size	Insurer Count ^a	Member Count	Direct Earned Premium	Market Share	Loss Ratio	Premium PMPM ^b
Small Group (1-50)	14	13,831	\$22,172,053	5.7%	82.7	\$126
Large Group (51-100)	16	13,461	\$15,027,508	3.8%	120.0	\$94
Large Group (101 +)	33	621,457	\$353,926,365	90.5%	101.7	\$57
Total Stop-Loss	40	648,749	\$391,125,926	100.0%	101.3	\$60

Data Source: Utah Accident & Health Survey

Note: Estimates may not total exactly due to rounding.

^a Insurer count column does not add up to total because an insurer may have more than one group type.

^b Direct earned PMPM

Small Group reported the highest premium PMPM. Large Group (51 to 100) reported a higher premium PMPM than Large Group (101 or more). These higher premiums are probably due to differences in stop-loss coverage attachment points, with Small Group and Large Group (51 to 100) reporting lower specific attachment points (see Table 31). Specific stop-loss is often more expensive than aggregate stop-loss and accounts for more of the premium. Large Group (101 or more) typically includes stop-loss coverage with higher specific and aggregate attachment points, which is consistent with their lower premium PMPM.

Stop-Loss Insurance Market by Attachment Points

Stop-loss insurance includes two types of coverage, specific and aggregate. These two types of coverage work together to protect an employer sponsored self-funded plan: specific stop-loss provides protection against the severity of unexpected claims, and aggregate stop-loss provides protection against the frequency of unexpected claims.

Specific Stop-Loss. Specific stop-loss (also known as individual stop-loss) provides protection for the employer sponsored self-funded plan against large claims costs for a single covered individual. Specific stop-loss coverage includes a limit, or attachment point, when a stop-loss policy will pay for a claim. The attachment point (also known as individual stop-loss deductible) is the dollar amount where the insurer reimburses the employer sponsored self-funded plan.

Table 31. Stop-Loss Membership by Specific Attachment Points in 2024

Attachment Point	Small Group (1 to 50)		Large Group (51 to 100)		Large Group (101 or more)		Total	
	Member Count ^a	Percent of Total	Member Count	Percent of Total	Member Count	Percent of Total	Member Count	Percent of Total
NONE	1,691	<0.3%	2,264	<0.3%	5,089	0.8%	9,044	1.4%
\$10,000	636	0.1%	102	<0.1%	246	<0.1%	984	0.2%
\$20,000	562	0.1%	178	<0.1%	5,727	0.9%	6,467	1.0%
\$30,000	3,678	0.6%	944	0.1%	10,469	1.6%	15,091	2.3%
\$40,000	407	0.1%	1,224	0.2%	7,725	1.2%	9,356	1.4%
\$50,000	1,423	0.2%	1,034	0.2%	39,706	6.1%	42,163	6.5%
\$60,000	36	<0.1%	535	0.1%	3,930	0.6%	4,501	0.7%
\$70,000	-	-	207	<0.1%	8,072	1.2%	8,279	1.3%
\$80,000	-	-	1	<0.1%	5,612	0.9%	5,613	0.9%
\$90,000	-	-	417	0.1%	3,334	0.5%	3,751	0.6%
\$100,000	5,398	0.8%	6,549	1.0%	132,242	20.4%	144,189	22.2%
\$200,000	-	-	3	<0.1%	125,084	19.3%	125,087	19.3%
\$300,000	-	-	-	-	76,921	11.9%	76,921	11.9%
\$400,000	-	-	3	<0.1%	30,177	4.7%	30,180	4.7%
\$500,000	-	-	-	-	46,998	7.2%	46,998	7.2%
\$600,000	-	-	-	-	20,121	3.1%	20,121	3.1%
\$700,000	-	-	-	-	36,063	5.6%	36,063	5.6%
\$800,000	-	-	-	-	1,002	5.6%	1,002	5.6%
\$900,000	-	-	-	-	4,066	0.6%	4,066	0.6%
\$1,000,000	-	-	-	-	49,720	7.7%	49,720	7.7%
\$2,000,000 or more	-	-	-	-	9,153	1.4%	9,153	1.4%
Total	13,831	2.1%	13,461	2.1%	621,457	95.8%	648,749	100.0%

Data Source: Utah Accident & Health Survey

Note: Estimates may not add up exactly to totals due to rounding.

^a Under Utah Code § 31A-43-301, a small group stop-loss policy is required to have a specific attachment point that is at least \$10,000.

Aggregate Stop-Loss. Aggregate stop-loss protects a self-funded employer group against an unusually high level of claim costs that affect the entire employer group. Under a typical stop-loss policy, the aggregate attachment point (an aggregate deductible or aggregate attachment factor) is the point at which the stop-loss insurer begins to pay for eligible medical expenses during a given contract period. This point is commonly referred to as an aggregate margin. This is usually shown as a percentage of expected claims. For example, an attachment point of 125 percent means the stop-loss coverage starts to pay when the percentage of excess claims reaches 25 percent above the 100 percent of expected claim costs. In 2024, most (59 percent) of the stop-loss membership included some kind of aggregate stop-loss coverage, with the rest (41percent) reporting “none.”

Table 32. Stop-Loss Membership by Aggregate Attachment Points for 2024

Attachment Point	Small Group (1 to 50)		Large Group (51 to 100)		Large Group (101 or more)		Total	
	Member Count ^a	Percent of Total	Member Count	Percent of Total	Member Count	Percent of Total	Member Count	Percent of Total
NONE	68	<0.1%	521	0.1%	265,635	40.9%	268,802	41.4%
85% to 89%	2,577	0.4%	-	-	22,574	3.5%	22,574	3.5%
90% to 94%	-	-	-	-	-	-	-	-
95% to 99%	28	<0.1%	-	-	-	-	28	<0.1%
100% to 104%	6,903	1.1%	8,643	1.3%	30,974	4.8%	46,519	7.2%
105% to 109%	3138	0.5%	360	0.1%	14826	2.3%	18,324	2.8%
110% to 114%	115	<0.1%	113	<0.1%	4,668	0.7%	4,896	0.8%
115% to 119%	337	0.1%	182	<0.1%	29,370	4.5%	29,889	4.6%
120% to 124%	511	0.1%	2088	0.3%	194,415	30.0%	197,014	30.4%
125% to 129%	154	<0.1%	891	0.1%	58,510	9.0%	59,555	9.2%
130% or more	-	-	663	0.1%	485	0.1%	1,148	0.2%
Total	13,831	2.1%	13,461	2.1%	621,457	95.8%	648,749	100.0%

Data Source: Utah Accident & Health Survey

Note: Estimates may not add up exactly to totals due to rounding.

Utah's Long-Term Care Insurance Market

Long-term care insurance is designed to provide specialized insurance coverage for skilled nursing care and custodial care in a nursing home, assisted living facility, or at home. Long-term care insurance typically covers specialized services that are not usually covered by comprehensive or major medical health insurance.

Long-term care insurance accounts for .4 percent of the commercial health insurance market in Utah (see Table 3). Long-term care insurers provided coverage for 28,244 members, or approximately .1 percent of Utah residents, in 2024. These estimates only refer to commercial long-term care insurance regulated by the Department. They do not include other types of long-term care coverage offered by self-funded employers or government programs. This section summarizes various aspects of the market including state of domicile, group size, and age. Enrollment in long-term care insurance is continually declining due to a shift in marketing and product availability. Many insurers are making long-term care benefits an option for life insurance policies rather than as stand-alone products.

Long-Term Care Market by Domicile

Foreign insurers provide nearly all of Utah's long-term care insurance. Fifty-nine foreign insurers account for almost all of the market (99.9 percent), with only one domestic insurer providing long-term care coverage (see Table 33).

Table 33. Total Long-Term Care Market by Domicile for 2024

Domicile	Insurer Count	Member Count	Direct Earned Premium	Market Share	Loss Ratio
Domestic	1	38	\$28,585	0.1%	636.2
Foreign	59	28,206	\$42,711,114	99.9%	109.4
Total	60	28,244	\$42,739,699	100.0%	109.7

Data Source: Utah Accident & Health Survey

Long-Term Care Market by Group Size

Long-term care plans are sold either as an individual or a group policy. Individual policies are sold directly to individual consumers. Group policies are sold as a single policy to a group of individuals, such as a group of employees, or an association plan.

Most long-term care insurers reported individual business, while only 25 of the 60 companies reported group business. Loss ratios were lower for group policies than for individual policies (see Table 34).

Table 34. Total Long-Term Care Market by Group Size for 2024

Group Size	Insurer Count ^a	Member Count	Direct Earned Premium	Market Share	Loss Ratio
Individual	51	14,528	\$27,423,412	64.2%	134.3
Group	25	13,716	\$15,316,287	35.8%	75.3
Total	60	28,244	\$42,739,699	100.0%	109.7

Data Source: Utah Accident & Health Survey

^a Insurer count column does not add up to total because an insurer may have more than one group size.

Long-Term Care Market by Age

As Utah's population has grown, the number of individuals over the age of 65 has increased. As people age, the cost of health care increases. As a result, long-term care insurance is important for older Utahns.

Commercial health insurers reported 28,244 members with long-term care insurance in Utah in 2024. About 34 percent of the members were under age 65 and the rest were 65 or older (see Table 35).

Table 35. Long-Term Care Membership by Age for 2024

Age	Member Count	Percent
Age 0-59	7,140	25.3%
Age 60-64	2,553	9.0%
Age 65-69	3,418	12.1%
Age 70-74	4,513	16.0%
Age 75-79	4,365	15.5%
Age 80-84	3,225	11.4%
Age 85+	3,030	10.7%
Total Members	28,244	100.0%

Data Source: Utah Accident & Health Survey

Note: Estimates may not total exactly due to rounding.

Utah’s Medicare Product Market

Medicare Supplement and Medicare Advantage policies are specialized health insurance products. Medicare Supplement (also known as Medigap) policies are sold as a “supplement” to the basic Medicare Part A (Hospital) and Part B (Medical) programs and provide additional coverage beyond the Medicare benefits. Medicare Advantage (also known as Medicare Part C) policies are sold as full replacement products. In other words, instead of providing specialized coverage for the “gaps” in Medicare (with Medicare still bearing most of the insurance risk), Medicare Advantage products offer an alternative to traditional Medicare and the health insurer bears the full risk of financial loss.

Another specialized insurance product is Medicare Part D. It allows health insurers to offer stand-alone pharmacy coverage. These plans provide coverage for prescription drugs, a medical benefit that Medicare Parts A and B do not normally cover. Medicare Part D is also included in many Medicare Advantage policies.

Medicare Supplement and Medicare Advantage products together account for 32 percent of Utah’s accident & health insurance market, with approximately 2 percent in Medicare Supplement and 30 percent in Medicare Advantage. Approximately 9 percent of Utah residents had coverage under a Medicare Supplement or Medicare Advantage product. Medicare Part D products account for about .5 percent of Utah’s accident & health insurance market and provide coverage for approximately 2 percent of Utahns.

These estimates only refer to commercial Medicare products offered in Utah’s commercial health insurance market. They do not include other types of Medicare products offered by self-funded employers or government programs. This section summarizes various aspects of the market, including state of domicile, age and gender demographics, and plan type.

Medicare Supplement by Domicile. The majority of Utah’s Medicare Supplement coverage is provided by foreign insurers. Foreign insurers account for nearly 68 percent of the market with 92 companies, and 6 domestic insurers provide the remaining 32 percent (see Table 36). Loss ratios were higher for the foreign insurers than for the domestic insurers.

Table 36. Total Medicare Supplement Market by Domicile for 2024

Domicile	Insurer Count	Member Count	Direct Earned Premium	Market Share	Loss Ratio
Domestic	6	24,009	\$69,969,052	32.1%	85.3
Foreign	92	55,776	\$147,706,548	67.9 %	82.5
Total	98	79,785	\$217,675,600	100.0%	83.4

Data Source: Utah Accident & Health Survey

Medicare Advantage by Domicile. Utah’s Medicare Advantage market has more domestic than foreign business, with 68 percent of the coverage provided by domestic insurers, and the remaining 32 percent provided by foreign insurers (see Table 37).

Table 37. Total Medicare Advantage Market by Domicile for 2024

Domicile	Insurer Count	Member Count	Direct Earned Premium	Market Share	Loss Ratio
Domestic	8	173,215	\$2,300,563,935	68.0%	87.1
Foreign	7	75,447	\$1,085,052,305	32.0%	90.8
Total	15	248,662	\$3,385,616,240	100.0%	88.3

Data Source: Utah Accident & Health Survey

Medicare Part D by Domicile. Foreign insurers provide Medicare Part D coverage for 93 percent of Utah’s market. Domestic insurers provide the remaining 7 percent (see Table 38).

Table 38. Total Medicare Part D Market by Domicile for 2024

Domicile	Insurer Count	Member Count	Direct Earned Premium	Market Share	Loss Ratio
Domestic	2	3,659	\$4,362,064	7.0%	93.5
Foreign	4	58,779	\$57,548,272	93.0%	91.2
Total	6	62,438	\$61,910,336	100.0%	91.3

Data Source: Utah Accident & Health Survey

Medicare Products by Age

Medicare Supplement policies, Medicare Advantage products, and Medicare Part D drug plans are generally designed for people age 65 and older. Medicare is offered earlier if you have a disability, End-Stage Renal Disease (permanent kidney failure requiring dialysis or a transplant), or ALS (also called Lou Gehrig's disease). Utah does not require insurers to offer Medicare Supplements to those under age 65 with Medicare, but Medicare Advantage Plans are available (with both Part A and Part B), and Medicare Part D is available.

In 2024, 98 commercial health insurers reported covering members with Medicare Supplement, 15 insurers offered Medicare Advantage, and 6 insurers provided Medicare Part D (see Tables 36, 37, 38). Through those insurers, 79,785 members were enrolled in Medicare Supplement coverage, 248,662 in Medicare Advantage, and 62,438 in Medicare Part D. With each of these, nearly all residents with coverage were over age 65 (see Tables 39, 40, 41).

Table 39. Medicare Supplement Membership by Age for 2024

Age	Member Count	Percent
Age 0-64	826	1.0%
Age 65 and Older	78,959	99.0%
Total Members	79,785	100.0%

Data Source: Utah Accident & Health Survey

Table 40. Medicare Advantage Membership by Age for 2024

Age	Member Count	Percent
Age 0-64	14,941	6.0%
Age 65 and Older	233,721	94.0%
Total Members	248,662	100.0%

Data Source: Utah Accident & Health Survey

Table 41. Medicare Part D Membership by Age for 2024

Age	Member Count	Percent
Age 0-64	3,547	5.7%
Age 65 and Older	58,891	94.3%
Total Members	62,438	100.0%

Data Source: Utah Accident & Health Survey

Medicare Products by Plan Type

Medicare Supplement Membership by Plan Type. Commercial health insurers reported 79,785 Medicare Supplement members in Utah in 2024 (see Table 42). They reported members across 17 Standardized Medicare Supplement plans or Pre-Standardized plans (plans in force prior to the federal government standardizing the plans that can be offered).

Table 42. Medicare Supplement Membership by Plan Type for 2024

Plan Type	Member Count	Percent
Plan A	273	0.3%
Plan B	115	0.1%
Plan C	785	1.0%
Plan D	337	0.4%
Plan E	71	0.1%
Plan F	14,853	18.6%
Plan F (High Deductible Plan)	14,817	18.6%
Plan G	27,594	34.6%
Plan G (High Deductible Plan)	11,142	14.0%
Plan H	128	0.2%
Plan I	110	0.1%
Plan J	556	0.7%
Plan J (High Deductible Plan)	558	0.7%
Plan K	349	0.4%
Plan L	188	0.2%
Plan M	331	0.4%
Plan N	7,372	9.2%
Pre-Standardized Plans	206	0.3%
Total Members	79,785	100.0%

Data Source: Utah Accident & Health Survey

Note: Estimates may not total exactly due to rounding.

The 5 most common Medicare Supplement plans were Plan G with 34.6 percent of membership, followed by Plan F at 18.6 percent, Plan F (High Deductible Plan) with 18.6 percent, Plan G (High Deductible Plan) with 14.0 percent, and Plan N with 9.2 percent. All other plans had 1 percent of the membership or less (see Table 42). Plans E, H, I, J, and J (High Deductible Plan) were discontinued and were no longer available to Medicare beneficiaries eligible after June 10, 2010. Plans C, F, and F (High Deductible Plan) were also discontinued and are no longer available to Medicare beneficiaries eligible as of January 1, 2020.

Medicare Advantage Membership by Plan Type. Commercial health insurers reported 248,662 Medicare Advantage members in Utah in 2024. Medicare Advantage plans (which are offered as alternatives to traditional Medicare, and bear the full risk of loss) are available in one

of five major plan types, shown below in Table 43.

In 2024, most of the membership was covered under an HMO plan, with just under 65 percent. The second most common was a PPO plan, with about 24 percent of the membership. Private FFS plans accounted for nearly 12 percent, and Medical Savings Accounts accounted for less than 1 percent. There was no one enrolled in the Special Need Plan in 2024.

Table 43. Medicare Advantage Membership by Plan Type for 2024

Plan Type	Member Count	Percent
Private Fee-for-Service	28,711	11.5%
Preferred Provider Organization	58,746	23.6%
Health Maintenance Organization	161,086	64.8%
Medical Savings Account	0	0.0%
Special Needs Plan	101	0.0%
Total Members	248,662	100.0%

Data Source: Utah Accident & Health Survey

Note: Estimates may not total exactly due to rounding.

Pharmacy Benefit Manager Rebates

Utah Code § 31A-46-301 and Utah Admin. Code R590-282 requires every licensed pharmacy benefit manager (PBM) operating in Utah to submit a report to the Insurance Department on or before April 1, 2020, and each year thereafter.

Each licensed PBM is required to report the aggregated total value of all rebates and administrative fees of contracted insurers. They must also report the percentage of aggregate rebates that the PBM retained under their agreement to provide pharmacy benefits management services to a contracting insurer.

The Department is required to publish this information in a manner that does not make a specific submission from a contracting insurer or pharmacy benefit manager identifiable, or disclose information that is a trade secret as defined in Utah Code § 13-24-2 (see Utah Code § 31A-46-301).

There were 49 licensed PBMs in Utah in 2024. Table 44 summarizes the information reported to the Department. Among these companies, 24 companies reported data and were actively doing business in Utah in 2024. Based on these reports, the overall rebate retention rate was 3.8 percent.

Table 44. Pharmacy Benefit Manager Rebates and Administrative Fees for 2024

Plan Type	Count	Total Rebates	Total Retained Percent Rebates Retained	Total Admin. Fees
PBMs actively doing business	24	\$429,507,717	\$16,242,829 3.8%	\$93,078,997
PBMs with no business	25	-	-	-
Total	49	\$429,507,717	\$16,242,829 3.8%	\$93,078,997

Data Source: Utah Pharmacy Benefit Manager Report

Conclusion

The Utah Insurance Department has regulatory authority over the commercial health insurance industry across Utah. This type of health insurance is critical to the health and wellbeing of Utahns who do not have employer sponsored or government sponsored health insurance. This report has detailed the strength of Utah's commercial health insurance market in 2024 through a variety of measures.

Nearly 42 percent of Utah's commercial health insurance market is comprehensive health insurance (also known as major medical). Comprehensive health insurance membership increased slightly from 2023, covering just over 24 percent of Utah residents.

In addition to regulating the commercial health insurance market in Utah, the Department also fields consumer complaints and launches investigations into allegations when necessary. In 2024, the number of complaints increased by 39 percent over 2023. The number of independent reviews also increased by 69 percent. This is a significant portion of the workload for the Department.

As the population of Utah continues to grow, so does the number of people who rely on the Department to regulate a critical industry within our state. Based on the findings in this report, as well as the Utah and Nationwide trends of the insurance industry, the Department has made four recommendations to the legislature to help ensure that all Utahns are treated equitably within this industry (see page 50).

References

- American Academy of Actuaries. (2018). Prescription drug spending in the U. S. healthcare system: An actuarial perspective (Issue Brief March 2018). American Academy of Actuaries: Washington, DC.
- American Rescue Plan Act. (2021). American Rescue Plan Act of 2021. Public Law No. 117–2. Available at: <http://uslaw.link/citation/us-law/public/117/2>.
- Barnett, J. C., and Berchick, E. R. (2017). Current Population Reports, P60-260, Health Insurance Coverage in the United States: 2016, U.S. Government Printing Office: Washington, DC.
- Berchick, E. R., Barnett, J. C., and Upton, R. D. (2019). Current Population Reports, P60-267, Health Insurance Coverage in the United States: 2018, U.S. Government Printing Office: Washington, DC.
- Berchick, E. R., Hood, E., and Barnett, J. C. (2018). Current Population Reports, P60-264, Health Insurance Coverage in the United States: 2017, U.S. Government Printing Office: Washington, DC.
- Centers for Medicare and Medicaid Services. (2018). Early 2018 Effectuated Enrollment SnapShot. Baltimore, MD: Centers for Medicare and Medicaid Services.
- Centers for Medicare and Medicaid Services. (2022). National Health Expenditures 2020 Highlights. Baltimore, MD: Centers for Medicare and Medicaid Services.
- Centers for Medicare and Medicaid Services. (2023). Early 2023 and Full Year 2022 Effectuated Enrollment Report. Baltimore, MD: Centers for Medicare and Medicaid Services.
- Claxton, G., Rae, M., Panchal, N., Whitmore, H., Damico, A., and Kenward, K. (2014). Health benefits in 2014: Stability in premiums and coverage for employer-sponsored plans. Health Affairs, 33 (10), 1851-1860.
- Claxton, G., Rae, M., Panchal, N., Whitmore, H., Damico, A., Kenward, K., and Long, M. (2015). Health Benefits in 2015: Stable trends in the employer market. Health Affairs, 34 (10), 1779-1788.
- Conway, D., and Branch, B. (2022). American Community Survey Briefs, ACSBR-013, Health Insurance Coverage Status and Type by Geography: 2019 and 2021, U.S. Government Printing Office: Washington, DC.
- Conway, D., and Branch, B. (2023). American Community Survey Briefs, ACSBR-015, Health Insurance Coverage Status and Type by Geography: 2021 and 2022, U.S. Government Printing Office: Washington, DC.

- Families First Coronavirus Response Act. (2020). Families First Coronavirus Response Act. Public Law No. 116-217. Available at: <http://uslaw.link/citation/us-law/public/116/127>.
- Hartman, M., Martin, A. B., Benson, J., Catlin, A., and the National Health Expenditure Accounts Team. (2020). National health care spending in 2018: Growth driven by accelerations in Medicare and private insurance spending. Health Affairs, 39 (1), 8-17.
- Hartman, M., Martin, A. B., Espinosa, N., Catlin, A., and the National Health Expenditure Accounts Team. (2018). National health care spending in 2016: Spending and enrollment growth slow after initial coverage expansions. Health Affairs, 37 (1), 1-11.
- Hartman, M., Martin, A. B., Washington, B., Catlin, A., and the National Health Expenditure Accounts Team. (2022). National health care spending in 2020: Growth driven by federal spending in response to the COVID-19 Pandemic. Health Affairs, 41 (1), 13-25.
- Institute of Medicine. (2012). Best care at lower cost: The path to continuously learning health care in America. Washington, DC: National Academies Press.
- Kaiser/HRET. (2015). Employer Health Benefits: 2014 Annual Survey. Menlo Park, CA: Kaiser Family Foundation/Health Research and Education Trust.
- Kaiser/HRET. (2016). Employer Health Benefits: 2015 Annual Survey. Menlo Park, CA: Kaiser Family Foundation/Health Research and Education Trust.
- Kaiser Family Foundation. (2023a). Employer Health Benefits: 2022 Annual Survey. San Francisco, CA: Kaiser Family Foundation.
- Kaiser Family Foundation. (2023b). Premium and workers contributions data from the Employer Health Benefits: 2022 Annual Survey. Available at: <https://www.kff.org/interactive/premiums-and-worker-contributions/>
- Keehan, S. P., Fiore, J. A., Poisal, J. A., Cuckler, G. A., Sisko, A. M., Smith, S. D., Madison, A. J., and Rennie, K. E. (2023). National Health Expenditure Projections, 2022-31: Growth to stabilize once the COVID-19 public health emergency ends. Health Affairs, 42 (7), 886-898.
- Keisler-Starkey, K., and Bunch, L. N. (2020). Current Population Reports, P60-271, Health Insurance Coverage in the United States: 2019, U.S. Government Printing Office: Washington, DC.
- Keisler-Starkey, K., and Bunch, L. N. (2021). Current Population Reports, P60-274, Health Insurance Coverage in the United States: 2020, U.S. Government Printing Office: Washington, DC.

- Martin, A. B., Hartman, M., Benson, J., Catlin, A., and the National Health Expenditure Accounts Team. (2016). National health spending in 2014: Faster growth driven by coverage expansion and prescription drug spending. Health Affairs, 35 (1), 150-160.
- Martin, A. B., Hartman, M., Benson, J., Catlin, A., and the National Health Expenditure Accounts Team. (2023). National health care spending in 2021: Decline in federal spending outweighs greater use of health care. Health Affairs, 42 (1), 6-17.
- Martin, A. B., Hartman, M., Lassman, D., Catlin, A., and the National Health Expenditure Accounts Team. (2021). National health care spending in 2019: Steady growth for the fourth consecutive year. Health Affairs, 40 (1), 14-24.
- Martin, A. B., Hartman, M., Washington, B., Catlin, A., and the National Health Expenditure Accounts Team. (2019). National health care spending in 2017: Growth slows to post-great recession rates; Share of GDP stabilizes. Health Affairs, 38 (1), 96-106.
- No Surprises Act. (2021). No Surprises Act included in the Consolidated Appropriations Act. Public Law No. 116-260. Available at: <http://uslaw.link/citation/us-law/public/116/260>.
- Patient Protection Affordable Care Act. (2010). Public Law No. 111–148 Section 5000A(d)(2)(b). Available at: <http://uslaw.link/citation/us-law/public/111/148>.
- PriceWaterhouseCoopers. (2017). Medical cost trend: Behind the numbers 2018. Washington, DC: PriceWaterhouseCoopers’ Health Research Institute.
- PriceWaterhouseCoopers. (2018). Medical cost trend: Behind the numbers 2019. Washington, DC: PriceWaterhouseCoopers’ Health Research Institute.
- Shrank, W. H., Rogstad, T. L., and Parekh, N. (2019). Waste in the U.S. health care system: estimated costs and potential for savings. JAMA, 322 (15), 1501-1509.
- Utah Economic Council. (2022). 2022 Economic Report to the Governor. Salt Lake City, UT: David Eccles School of Business and Governor’s Office of Management and Budget.
- Utah Insurance Department. (2017). 2018 Utah Rates: Individual and Small Group. Available at: <https://le.utah.gov/interim/2017/pdf/00004290.pdf>

Appendix

Recommendations

As requested by the Utah Legislature, the Insurance Department has developed a list of recommendations for legislative action that have the potential to improve Utah's health insurance market.

- 1) Revise multiple health insurance reports due dates to a single date to create efficiencies and cost savings for the State of Utah and insurers.
- 2) Require all insurers offering Medigap insurance to extend product offerings to Medicare recipients who are eligible by disability.
- 3) Improve education and training on the nature of health care and health insurance costs in State consumer and financial education curriculum standards, with an emphasis on teaching consumers how to spend less and get more value out of their health care purchases.

Table 45. List of Comprehensive Health Insurers

Company Name	State of Domicile	Utah Premiums	Market Share	Loss Ratio
SelectHealth, Inc.	UT	\$2,962,850,457	64.2%	84.5
Regence BlueCross BlueShield of Utah	UT	\$510,608,171	11.1%	87.8
Aetna Health of Utah Inc.	UT	\$328,589,290	7.1%	98.5
UnitedHealthcare Insurance Company	CT	\$257,835,491	5.6%	77.9
Cigna Health & Life Insurance Company	CT	\$159,402,565	3.5%	85.6
Aetna Life Insurance Company	CT	\$152,553,383	3.3%	75.4
University of Utah Health Insurance Plans	UT	\$122,152,583	2.6%	93.0
Health Care Service Corporation, a Mutual Legal Reserve Company	IL	\$49,372,187	1.1%	84.5
Molina Healthcare of Utah, Inc.	UT	\$38,452,290	0.8%	91.8
Balanced Insurance Company of Utah (FKA Angle Insurance Company of Utah)	UT	\$11,832,200	0.3%	80.1
WMI Mutual Insurance Company	UT	\$5,317,838	0.1%	68.1
UnitedHealthcare of Utah, Inc.	UT	\$5,210,177	0.1%	82.6
State Farm Mutual Automobile Insurance Company	IL	\$3,347,234	0.1%	75.0
All Savers Insurance Company	IN	\$2,541,881	0.1%	-39.5
4 Ever Life Insurance Company	IL	\$2,093,410	0.0%	71.2
MotivHealth Insurance Company	UT	\$1,202,935	0.0%	104.2
Bridgespan Health Company	UT	\$1,015,945	0.0%	99.1
Imperial Healthplan of the Southwest, Inc	UT	\$308,586	0.0%	0.0
Educators Health Plans Life, Accident and Health, Inc.	UT	\$253,000	0.0%	253.7
American National Life Insurance Company of Texas	TX	\$179,484	0.0%	152.9
Equitable Financial Life Insurance Company	NY	\$13,477	0.0%	0.0
Standard Life & Accident Insurance Company	TX	\$6,680	0.0%	-2.3
The Prudential Insurance Company of America	NJ	\$6,189	0.0%	4224.3
Connecticut General Life Insurance Company	CT	\$720	0.0%	28.3
Transamerica Life Insurance Company	IA	\$325	0.0%	0.0
Metropolitan Life Insurance Company	NY	\$258	0.0%	0.0
Humana Insurance Company	WI	-\$157,544	0.0%	-58.4
Total Comprehensive Health Insurance	27	\$4,614,989,212	100.0%	85.4

List of Health Insurance Mandates in Utah

Coverage Mandates

Required by Federal statute:

1. Dependent coverage from the moment of birth or adoption (31A-22-610, Health Insurance Portability and Accountability Act)
2. Coverage through a noncustodial parent (31A-22-610.5; Social Security Act)
3. Open enrollment for child coverage ordered by a court (31A-22-610.5; Social Security Act)
4. Medicare supplement insurance, including a preexisting condition provision (31A-22-620; NAIC Standard; Title XVIII of the Social Security Amendment)
5. Individual and small group guaranteed renewability (31A-22-618.6; 31A-22-618.7; Health Insurance Portability and Accountability Act; Patient Protection and Affordable Care Act)
6. Prohibition of preexisting conditions exclusions (31A-1-301; 31A-22-605.1; Health Insurance Portability and Accountability Act; Patient Protection and Affordable Care Act)
7. Small group eligibility and individual guaranteed issue (31A-30-108; Health Insurance Portability and Accountability Act; Patient Protection and Affordable Care Act)
8. Maternity coverage requirements on groups of 15 or more employees (Pregnancy Discrimination Act; Patient Protection and Affordable Care Act)
9. COBRA benefits for employees of an employer with 20 or more employees (Consolidated Omnibus Budget Reconciliation Act)
10. Preexisting condition limitations (31A-22-605.1; Health Insurance Portability and Accountability Act; Patient Protection and Affordable Care Act)
11. Limitation of annual and lifetime limits for essential health benefits (Patient Protection and Affordable Care Act)
12. Coverage of preventative health services (Patient Protection and Affordable Care Act)
13. Eligibility of children until at least age 26, including married children (31A-22-610.5; Patient Protection and Affordable Care Act, 2010)
14. Coverage for participation in approved clinical trials (Patient Protection and Affordable Care Act)
15. Essential health benefits and actuarial value requirements for comprehensive health insurance coverage (Patient Protection and Affordable Care Act)

Required by State statute:

1. Minimum policy standards (31A-22-605)
2. Extension of coverage for a dependent child with a disability (31A-22-611)
3. Mini-COBRA benefits for employees of an employer with less than 20 employees (31A-22-722)
4. Provisions pertaining to armed forces (31A-22-717) Court order coverage for minor children

- outside the service area (31A-45-401)
- 5. Access to health care including a rural county (31A-45-501)
- 6. Adverse benefit determination review process (31A-22-629)

Benefit Mandates

Required by Federal statute:

- 1. Maternity stay minimum limits (31A-22-610.2; Newborns' & Mothers' Health Protection Act)
- 2. Pediatric vaccine benefit level (31A-22-610.5, Omnibus Budget Reconciliation Act)
- 3. Coverage of a mental health condition or substance use disorder (31A-22-625; Mental Health Parity and Addiction Equity Act; Patient Protection and Affordable Care Act)
- 4. Coverage of emergency medical services (31A-22-627; Federal Patient Bill of Rights Plus Act, Patient Protection and Affordable Care Act)
- 5. Mastectomy provisions (31A-22-630; 31A-22-719; Women's Health & Cancer Rights Act)
- 6. Alcohol and drug dependency treatment (31A-22-645; Patient Protection and Affordable Care Act)

Required by State statute:

- 1. Indemnity benefit for adoption (31A-22-610.1)
- 2. Coordination of benefits (31A-22-619)
- 3. Dietary products for inborn metabolic errors (31A-22-623)
- 4. Access to primary care including OB/GYNs, pediatricians physician assistant (31A-22-624)
- 5. Diabetes coverage (31A-22-626)
- 6. Standing referral to a specialist (31A-22-628)
- 7. Coverage for prosthetic devices (31A-22-638)
- 8. Cancer treatment parity (31A-22-641)
- 9. Coverage for autism spectrum disorder (31A-22-642)

Provider Mandates

Required by Federal statute:

- 1. Adequate provider network in the individual and small employer market (Patient Protection and Affordable Care Act)

Required by State statute:

- 1. Network provider contract provisions (31A-45-303)
- 2. Managed care organization payments to noncontracting providers in rural areas (31A-45-501)

Statutory Requirements and Methods Overview

Statutory Requirements

Utah Code § 31A-2-201.2 requires that the Utah Insurance Department produce an annual evaluation of the health insurance market. The statutory requirements for this evaluation are shown below:

- (1) Each year the commissioner shall:
 - (a) conduct an evaluation of the state's health insurance market;
 - (b) report the findings of the evaluation to the Health and Human Services Interim Committee before December 1 of each year; and
 - (c) publish the findings of the evaluation on the department website.
- (2) The evaluation required by this section shall:
 - (a) analyze the effectiveness of the insurance regulations and statutes in promoting a healthy, competitive health insurance market that meets the needs of the state, and includes an analysis of:
 - (i) the availability and marketing of individual and group products;
 - (ii) rate changes;
 - (iii) coverage and demographic changes;
 - (iv) benefit trends;
 - (v) market share changes; and
 - (vi) accessibility;
 - (b) assess complaint ratios and trends within the health insurance market, which assessment shall include complaint data from the Office of Consumer Health Assistance within the department;
 - (c) contain recommendations for action to improve the overall effectiveness of the health insurance market, administrative rules, and statutes;
 - (d) include claims loss ratio data for each health insurance company doing business in the state;
 - (e) include information about pharmacy benefit managers collected under Section 31A-46-301; and
 - (f) include information, for each health insurance company doing business in the state, regarding:
 - (i) preauthorization determinations; and
 - (ii) adverse benefit determinations.
- (3) When preparing the evaluation and report required by this section, the commissioner may seek the input of insurers, employers, insured persons, providers, and others with an interest in the health insurance market.
- (4) The commissioner may adopt administrative rules for the purpose of collecting the data required by this section, taking into account the business confidentiality of the insurers.
- (5) Records submitted to the commissioner under this section shall be maintained by the commissioner as protected records under Title 63G, Chapter 2, Government Records Access and Management Act.

Methods Overview

This report primarily uses data from two sources: the NAIC Financial Database and the Utah Accident & Health Survey. It also uses information from national data sources and government agencies. The report will continue to evolve as required to meet the needs of the Utah Legislature.

Qualifications. The accuracy of the information in this publication depends on the quality of the data supplied by commercial health insurers. While the information presented here is believed to be correct and every effort has been made to obtain accurate information, the Insurance Department cannot control for variations in the quality of the data supplied by commercial health insurers or differences in how insurers interpret NAIC and Insurance Department data submission guidelines.

NAIC Financial Database. The NAIC Financial Database is a nationwide database maintained by the National Association of Insurance Commissioners. It contains data obtained from insurance companies' annual financial statements. The data summarizes the total accident & health premium and losses in Utah reported by commercial health insurers to the NAIC.

Utah Accident & Health Survey. The Utah Accident & Health Survey is submitted annually to the Insurance Department. All commercial health insurers are required to file this report. This survey provides detailed information on commercial insurance activity in Utah. It includes information that allows the Insurance Department to estimate trends in Utah's commercial health insurance market, including market share, number of covered lives, loss ratios, and cost of insurance. For this report, data from the years 2015 to 2024.

The survey includes several major components: accident & health insurance, stop-loss insurance, Medicare supplemental insurance, long term care insurance, administration of self-funded plans, as well as comprehensive health insurance. The comprehensive insurance section includes detailed information on plan types, group size, and year-end member months. This additional detail allows the Insurance Department to evaluate changes in the comprehensive health insurance market with much greater accuracy.

In 2010, the Utah Accident & Health Survey was reorganized and expanded to include more detailed measures of the comprehensive health insurance market including the Small Employer Defined Contribution Market, analysis of certain types of benefit plans, and measures of certain types of insurance code mandates.

In 2014, the Utah Accident & Health Survey was expanded to include more detailed measures of the comprehensive health insurance market including measures of ACA compliant and Non-ACA compliant plans, and the Federally Facilitated Marketplace (FFM).

The Utah Accident & Health Survey does not specifically measure differences in benefit structure, demographics, or the health status of the commercially insured population. Despite this limitation, this survey (along with the NAIC Financial Database) is a valuable source of data on

Utah's commercial health insurance market and as such provides useful information on commercial health insurance.

Loss Ratios vs MLR. The loss ratios used in this report differ from the NAIC medical loss ratio (MLR) methodology that adjusts for taxes and fees. This report uses the traditional loss ratio methodology, incurred claims divided by earned premium. The MLR methodology is designed for use with comprehensive health insurance business and cannot be applied to all other types of accident & health insurance. Using the traditional loss ratio allows us to compare all types of accident & health insurance.

Health Care Preauthorization Reporting. As required by Utah Code § 31A-22-650, the Insurance Department collects data on health care preauthorizations. Starting in 2020, comprehensive health insurers doing business in Utah are required to annually submit the Utah Adverse Preauthorization Survey to the Insurance Department. This survey provides detailed information on health care authorizations. Data was collected for the years 2020 through 2024.

Utah Pharmacy Benefit Manager Report. As required by Utah Code § 31A-46-301, the Insurance Department collects data on pharmacy drug rebates. Starting in 2019, Pharmacy Benefit Managers licensed in the State of Utah are required to annually submit the Utah Pharmacy Benefit Manager Report to the Insurance Department. This report provides detailed information on pharmacy drug rebates and administrative fees. Data was collected for the years 2019 through 2024.

Glossary

This section includes a brief glossary of some specialized terms used in this report, which may be unclear to readers who are unfamiliar with Utah's health insurance industry.

Commercial health insurance: Any type of accident or health insurance product sold by a commercial health insurer. It refers to any type of accident or health insurance product permitted under the Utah Insurance Code.

Commercial health insurer: An insurance company that is registered with the Utah Insurance Department and is licensed to sell any type of accident or health insurance product in the State of Utah.

Commercial insurance health benefit plan: Another name for comprehensive health insurance. See also Comprehensive health insurance and Comprehensive health insurer.

Comprehensive health insurance: A subset of commercial health insurance. A comprehensive plan is a general-purpose health insurance product that provides a broad range of insurance coverage for basic medical services typically provided by a physician, including hospital and medical services, and in most cases, durable medical equipment and drugs. Because of the wide variety of basic medical services it covers, these plans are frequently called "major medical", "comprehensive health", or "comprehensive hospital and medical" to distinguish them from other types of accident or health insurance products with more limited benefits. It is the insurance product most people think of when they hear the term "health insurance".

Comprehensive health insurer: A commercial health insurer that offers a comprehensive health insurance product.

Domestic insurer: An insurer licensed to sell insurance in Utah and which also has its home office in Utah. Insurance companies that have a home office in Utah are said to be "domiciled in Utah". The state of domicile is important because most of the direct regulation of individual insurance companies is done by the state where the company is domiciled (e.g., solvency requirements, etc). See also Foreign insurer.

Employer sponsored self-funded health benefit plan: The key feature of these plans is that the risk of loss is taken on by the sponsoring organization (e.g., a health benefit plan offered by a large employer or non-profit association group), rather than a commercial health insurer. These plans are exempt from state regulation under the Federal ERISA statute, as they are not considered the "business of insurance", but an employee benefit plan. Self-funded plans are regulated by the Federal Department of Labor and states have no regulatory authority over these plans.

Foreign insurer: An insurer licensed to sell insurance in Utah, but it does not have a home office in Utah. It is domiciled in another state. See also Domestic insurer.

Government sponsored health benefit plan: Any health benefit plan offered by a federal or state government agency, where the government bears the risk of loss. These plans include Medicare, Medicaid, Children’s Health Insurance Program (CHIP), Primary Care Network (PCN), and the Utah Comprehensive Health Insurance Pool (HIPUtah). The PCN program ended in 2019 and the HIPUtah program ended in 2014. These plans were not included in the 2024 Health Insurance Market Report and did not previously include any health benefit plans for government employees, which are considered employer sponsored self-funded health benefit plans. See also Employer sponsored self-funded health benefit plans.